

DEPARTMENT OF THE ARMY
HEADQUARTERS, UNITED STATES ARMY MEDICAL COMMAND
JBSA FORT SAM HOUSTON, TEXAS 78234-6000

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MEDCOM Regulation
No. 40-50

Medical Services
CAREER MANAGEMENT FIELD (CMF) 68
CLINICAL BASELINE COMPETENCIES FOR ENLISTED MEDICAL PERSONNEL
PERFORMING DIRECT PATIENT CARE AT THE MILITARY TREATMENT FACILITY

Supplementation of this regulation and establishment of forms other than MEDCOM forms are prohibited without prior approval from HQ MEDCOM, ATTN: MCHO-CL.

1. HISTORY. This issue publishes a major revision of this regulation and should be read in its entirety.

2. PURPOSE. This regulation provides policy, assigns responsibility, and identifies clinical baseline competency tasks for enlisted personnel who perform direct patient care and are identified under the CMF 68, military occupational specialty (MOS), and/or additional skill identifier (ASI). In addition, guidance presented assists the local commander in determining those patient care activities that require direct and indirect supervision and those activities that could be delegated.

3. REFERENCES.

- a. Army Regulation (AR) 40-68, Clinical Quality Management.
- b. AR 350-1, Army Training and Leader Development.
- c. AR 351-3, Professional Education and Training Programs of the Army Medical Department.
- d. AR 601-210, Active and Reserve Components Enlistment Program.
- e. Department of the Army Pamphlet (DA Pam) 611-21, Military Occupational Classification and Structure.
- f. Field Manual 4-02, Army Health System.
- g. Department of Defense Manual 6025.13, Medical Quality Assurance (MQA) and Clinical Quality Management in the Military Health System (MHS), 29 October 2013.

* This regulation supersedes MEDCOM Regulation 40-50, 6 January 2006

- h. The Joint Commission (TJC), Comprehensive Accreditation Manual.
- i. United States Army Medical Command (MEDCOM) Regulation 40-41, The Patient Safety Program.
- j. United States Special Operations Command Directive 350-29, Special Operations Forces (SOF) Baseline Interoperable Standards (SOFBIS) for Medical Training.
- k. Most Recent Soldier Training Publication (STP) for Respective MOSs.
- l. Training Circular (TC) 8-800, Medical Education and Demonstration of Individual Competence.

4. EXPLANATION OF ABBREVIATIONS AND TERMS. Abbreviations and special terms used in this regulation are explained in the glossary.

5. APPLICABILITY. This regulation applies to all CMF 68 Soldiers, within all Army components, who perform direct patient care duties within the military treatment facility (MTF), their supervisors, and the MTF commander.

NOTE: MOSs that do not provide direct patient care are still required to have skills competency based assessed in accordance with (IAW) AR 40-68. Those MOSs identified are 68A, 68J, 68R, 68S, and 68T.

6. RESPONSIBILITIES.

a. The commander, in determining how the MTF health care mission can best be met, will ensure:

(1) A Medical Director is identified to design and validate a training program consistent with the skills necessary for a 68W to operate independently in an austere environment. Additionally, the Medical Director will work with the various departments in the MTF to ensure supervisory and delegation roles as they pertain to the 68W are clearly defined and that they are consistent with legal and regulatory guidance.

(2) Each CMF 68 Soldier who performs direct patient care is used in accordance with his/her primary military occupational specialty (PMOS), his/her ASI, his/her demonstrated knowledge and technical proficiency, and the mission requirements of the organization to which the CMF 68 Soldier is assigned.

(3) Evaluation of the PMOS and/or ASI skill competency and unique mission requirements will be conducted as follows:

- (a) Upon arrival to the facility.

- (b) 6 months after arrival.
 - (c) On the 1-year anniversary after arrival.
 - (d) Annual thereafter.
- (4) IAW AR 351-3, priority of training will be established in the following order:
 - (a) AOC/MOS training conducted in the MTF.
 - (b) SI/ASI Training conducted in the MTF.
 - (c) Medical skills readiness training.
 - (d) Other Military training requirements.
 - (e) Civilian agreements.
- b. Executive leaders will ensure:
 - (1) Established mechanisms to sustain identified skills and provide additional training are maintained.
 - (2) Professional standards within the specialty area guide the frequency of skill-specific sustainment and competency of skills and treatments.
- c. The privileged, licensed, certified, and/or registered individual (i.e. physician, nurse practitioner, physician assistant or RN) will be accountable for the management and provision of patient care to include all decisions regarding delegation.
- d. CMF 68 Soldiers will be responsible for providing safe patient care within the limits of their documented competency, training, job description, PMOS and/or ASI, and the organization's mission requirements.
- e. MTF Education and Training Departments will ensure that-
 - (1) Orientation programs are developed that will prepare the 68 series Soldiers for duty within the MTF.
 - (2) A robust training program is instituted which ensures the development of medical Soldiers focusing the following:
 - (a) The development of skills that will allow the 68 series Soldier to operate within the MTF as independently as possible consistent with applicable laws and regulations.

(b) The training and sustainment of skills that while not utilized independently within the MTF, may be part of the Soldier's Critical Task List (CTL) contained in applicable Soldier Training Publications (STP).

7. PERFORMANCE GUIDELINES.

a. All CMF 68 Soldiers awarded an MOS and/or ASI will be employed as indicated by their technical training and demonstrated/validated skills within the MTF. References for baseline clinical competencies of each MOS/ASI who perform direct patient care are listed in the appendixes A through Y as follows:

Appendix A	68B	Orthopedic Specialist
Appendix B	68C	Practical Nurse Specialist
Appendix C	68CM3	Dialysis Specialty
Appendix D	68D	Operating Room Specialist
Appendix E	68E	Dental Specialist
Appendix F	68EN5	Dental Laboratory Specialty
Appendix G	68EX2	Preventive Dentistry Specialty
Appendix H	68F	Physical Therapy Specialist
Appendix I	68G	Patient Administration Specialist
Appendix J	68H	Optical Fabrication Specialist
Appendix K	68K	Medical Laboratory Specialist
Appendix L	68KM2	Cytotechnology
Appendix M	68L	Occupational Therapy Specialist
Appendix N	68M	Nutrition Care Specialist
Appendix O	68N	Cardiovascular Specialist
Appendix P	68P	Radiology Specialist
Appendix Q	68PM5	Nuclear Medicine Specialty
Appendix R	68Q	Pharmacy Specialist
Appendix S	68U	Ear, Nose, and Throat Specialist
Appendix T	68V	Respiratory Therapy Specialist
Appendix U	68W	Health Care Specialist
Appendix V	68WF2	Flight Paramedic
Appendix W	68X	Behavioral Health Specialist
Appendix X	68XM8	Drug and Alcohol Counseling
Appendix Y	68Y	Eye Specialist

NOTE: MOSs are designated as "specialist"; ASIs are designated as "specialty."

b. The MOS/ASI course programs receive accreditation from civilian equivalent training programs when available. If a civilian equivalent training program is not found, validation for established training standards are maintained by the designated MOS and/or ASI teaching department at the U.S. Army Medical Department Center and School, U.S. Army Health Readiness Center of Excellence (AMEDDC&S HRCOE).

c. All individual clinical baseline competencies of each direct patient care task identified in the CMF 68 MOS/ASI appendixes may be obtained by contacting the appropriate MOS and/or ASI teaching department at the AMEDDC&S HRCOE or the MOS enlisted subject matter expert.

(1) The skill and procedure guidelines reflect national standard guidelines or the equivalent critical task where applicable.

(2) Guidelines and Competency Skills Assessments may be supplemented locally. The written protocol must be concise and convey specific skills with acceptable standards and/or outcomes.

d. The CMF 68 Soldier who has been awarded the MOS/ASI will be permitted to perform the identified clinical skills and/or tasks, attending additional formalized training at the local unit. Initial and on-going competency assessment and/or skill validation/certification are appropriate prior to any procedure being performed.

(1) Leaders will ensure all competency assessments are documented on the appropriate form in the Competency Assessment Folder (CAF)

(2) Competency Based forms can be supplemented with additional tasks as the mission dictates. Preprinted and standardized Competency Based forms are not all inclusive, but can be used as a starting point.

e. Training programs are required for unit-specific or age-specific clinical competencies and/or to acquire advanced knowledge/skills proficiency. This training will include didactic content and clinical experience.

f. The CMF 68 Soldiers identified in this regulation who are working in a MTF, in any capacity (e.g., assigned, attached, etc) for proficiency or sustainment training will function under direct supervision until they are able to demonstrate competence in the performance of applicable tasks related to their PMOS, ASI, or documented competency.

(1) The CMF 68 Soldier assigned to table of organization and equipment (TOE) units and selected for training will be given priority to practice within his/her MOS/ASI.

(2) Recommend the CMF 68 training be structured in conjunction with MEDCOM's Medical Skills Readiness Training program.

8. DELEGATION.

a. The supervisor may only delegate patient care activities that are within their own scope of practice and consistent with the commander's guidance as well as legal and regulatory requirements. Additionally, it is incumbent upon the delegator to ensure that the 68 series Soldier is properly trained and capable of performing the delegated task.

b. CMF 68 Soldiers to whom patient care activities are delegated may be supervised by a privileged, licensed, certified, and/or registered individual. However, the responsibility and accountability for the overall care provided and for associated patient outcomes remains with the delegating individual. .

c. All decisions related to delegation of patient care activities must be based on a concern for public protection and patient safety.

d. Professional judgment on the part of the delegating individual is required to determine which patient care activities and/or tasks are appropriate for delegation. As a baseline, activities appropriate for delegation meet the following criteria:

(1) Frequently or routinely reoccurring tasks have predictable results and minimal potential risk to the patient.

(2) Do not require the individual to exercise independent judgment.

(3) Do not require complex and/or multi-dimensional application of the clinical or nursing process.

(4) Use an established and unchanging procedure (i.e., protocol, Clinical Practice Guideline, or standing operating procedure).

e. Selected invasive and high-risk tasks/procedures (such as suturing or nail removal) may be performed by 68 series Soldiers who have received documented, formal training; such training may include a certification process. Local policy will direct which high-risk tasks/procedures may be delegated and to whom and what level of supervision is required. The privileged provider/professional who is responsible for direct/indirect supervision of the individual performing a given task/procedure is also responsible for the immediate post-procedure evaluation and disposition of the patient.

f. In the event that a particular task is cannot be delegated within an MTF due to legal or regulatory restrictions (i.e. blood or medication administration), a maximum amount of latitude will be given to allow for the training of the 68 series Soldier in order to prepare them for duty in austere environments. An example of such an instance might include setting up and assisting with the administration of blood up to the point at which the product is connected to the patient.

g. It is the responsibility of local leadership to ensure: that the individual competency of the 68 series Soldier is assessed; competency-based orientation is provided; and utilization of personnel and delegated tasks are based upon demonstrated knowledge, skills, and technical proficiency.

9. EVALUATIONS/OUTCOMES. Regardless of the setting or the educational preparation of the CMF 68 Soldier performing the procedure, a high-quality standard of care must be maintained throughout the organization. Accountability for decisions, that either directly or indirectly affects patient care delivery and patient care outcomes, remain with the organization's leadership. To ensure quality care is sustained, evaluation of patient outcomes will be included in the organizational performance improvement/risk management program. Patient care outcomes will be assessed, trended, analyzed, and have action plans to improve (as appropriate) included in the specific unit's/clinic's performance improvement program.

APPENDIX A - ORTHOPEDIC SPECIALIST (68B)

1. JOB DESCRIPTION. To assist the orthopedic surgeon and physician assistant in casting, splinting, and traction procedures, and the operation and maintenance of orthopedic equipment.

2. MILITARY COURSE. Phase 1: Medical Education and Training Campus (METC), JBSA Fort Sam Houston, TX; Phase 2: five sites.

a. Course length. 14 weeks (Phase 1: 8 weeks; Phase 2: 6 weeks).

b. Accreditation. No formal accreditation officially recognized by National Association of Orthopedic Technologist (NAOT) and by American Society of Orthopedic Professionals (ASOP) as a national school of training.

c. American Council on Education credit hours. 15 credit hours.

d. School affiliation. None.

e. Degree awarded. None.

f. Eligibility for certification or licensing. Eligible to take the National Board for Certification of Orthopedic Technologists (NBCOT) or the Registered Orthopedic Technician (ROT) upon graduation from course.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision. None.

b. Procedures requiring indirect supervision.

(1) Applies upper/lower extremity splints (double sugar tong, long arm, short arm volar, sugar tong, ulnar gutter, radial gutter, thumb spica, long leg, short leg, coaptation, medial lateral).

(2) Applies upper extremity casts (ulnar gutter, short arm radial gutter, short arm cobra, short arm thumb spica, long arm thumb spica, standard long arm, standard short arm, long arm cylinder, long arm hanging, short arm cast with finger splint, and outrigger, munster).

(3) Applies lower extremity casts (long leg, short leg, long leg cylinder, delbet, quadrilateral weight bearing, patella tendon weight bearing, hip spica).

(4) Applies specialty casts (Velpau, body jacket, Minerva jacket, shoulder brace).

(5) Applies traction (Bryant's, finger, Russell's, Buck's, pelvic, Dunlop's, balanced suspension, suspension with skin/skeletal traction, right angle skin/skeletal traction, and cervical traction).

(6) Performs debridement of wounds.

(7) Sets up orthopedic bed with trapeze.

(8) Conducts an inspection of orthopedic bed and equipment.

(9) Applies wedge and bivalve cast.

(10) Establishes a sterile pre-surgical field.

(11) Applies a compression dressing.

(12) Bandages stump of patient with amputation.

4. RELATED CIVILIAN TRAINING.

a. Course length. The most similar course is 1 year in length.

b. Degree awarded. Associate of Science.

c. Areas of clinical competency. There are no established national standards for training of orthopedic technicians currently published.

5. LICENSING/CREDENTIALING.

a. Army standard/requirement(s) for licensure and/or certification. None.

b. National and State requirements. Technicians must pass the national certification test the OTC offered by NBCOT or the ROT offered by ASOP. There are no State licensing requirements.

c. Additional requirements after completion of accredited course. Certifications are established by the NBCOT and ASOP in conjunction with NAOT (ASOP, PO BOX 4770, Seminole, and FL 33757). (NBCOT, Columbia Assessment Services, Inc., P.O. Box 14148, Research Triangle Park, NC 27709).

(1) 2 years on-the-job full-time experience in orthopedics.

(2) Completion of an orthopedic technologist school or program, plus 6 months full-time equivalent experience in orthopedics.

(3) Completion of recognized orthopedic technologist school or program

(4) Certified in an orthopedic-related field and a minimum of 1 years' experience in orthopedics.

d. Degree requirements. None.

e. Licensing/certification examination. National certification is offered by NBCOT.

6. CONTINUING EDUCATION REQUIREMENTS.

a. Army standard/requirement(s) for licensure and/or certification. None.

b. Maintain civilian license/certification. To maintain certification, orthopedic technicians must submit 120 continuing education hours to NBCOT every 6 years.

7. MOS REQUIREMENT – Introduction to Basic Army Medicine: Active Army/Reserve Component (AA/RC) Soldiers must attend the Introduction to Basic Army Medicine Course prior to attending the Orthopedic Specialist course. This is a 4 week program held at AMEDDC&S HRCOE.

APPENDIX B – PRACTICAL NURSE SPECIALIST (68C)

1. JOB DESCRIPTION. To provide safe and effective nursing care in both inpatient and outpatient settings; initiate and manage basic medical and nursing treatments for casualties/patients acquiring illness or injury during operational deployments and hospital settings; engage in effective communication; demonstrate leadership skills on the health care team as a provider and coordinator of care; function as first level NCOs; serve as wardmasters at MTFs where they manage personnel supplies and maintain equipment.

2. MILITARY COURSE. Phase 1: AMEDDC&S HRCOE, JBSA Fort Sam Houston, TX; Phase 2: five Sites.

a. Course length. 51 weeks. Phase 1: 11 weeks (Fort Sam Houston, TX), Phase 2: 40 weeks.

b. Accreditation. Annual approval by Texas Board of Nursing.

c. American Council on Education credit hours. The following semester hours are cumulative: 68C10 in the lower-division baccalaureate or associate's degree category. Clinical nursing experience. 41 credit hours.

d. School affiliation. None.

e. Degree awarded. None.

f. Eligibility for certification or licensing. The MOS must maintain basic health care provider cardiopulmonary resuscitation (CPR), IAW American Heart Association standards without a lapse in qualification.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision. None.

b. Procedures requiring indirect supervision.

- (1) Provides assessments and interventions as prescribed for patients.
- (2) Assists the health care team with resuscitation of a patient.
- (3) Collaborates with a health care team to implement direct patient care.
- (4) Prioritizes treatments and care of individual and multiple patients.
- (5) Provides instruction to restore health.
- (6) Administers and monitors parenteral therapy.
- (7) Administers, monitors, and records medications and their effects within scope of practice.

4. RELATED CIVILIAN TRAINING.

- a. Course length. 1 year.
- b. Degree awarded. Based upon structure of the Licensed Practical Nurse (LPN)/Licensed Vocational Nurse (LVN) course program.
- c. Areas of clinical competency. Texas Board of Nursing.

5. LICENSING/CREDENTIALING.

- a. Army standard/requirement(s) for licensure and/or certification. A current, active, valid, and unrestricted LVN/LPN license.
- b. National and State requirements. Course approval by Texas Board of Nursing.
- c. Additional requirements after completion of accredited course. None.
- d. Degree requirements. None.
- e. Licensing/certification examination. NCLEX-PN. Department of Defense Directive (DoD) Directive 6025.13, AR 601-210, and DA Pam 611-21.

6. CONTINUING EDUCATION REQUIREMENTS.

- a. Army standard/requirement(s) for licensure and/or certification. LPN licensure varies by State National Council Boards of Nursing Licensed Practical/Vocational Nurse requirements.
- b. Hours required to maintain license/certification. Varies by State National Council Boards of Nursing Licensed Practical/Vocational Nurse requirements.

7. MOS REQUIREMENT - Soldiers (AA/RC) must meet and maintain the requirements of the 68C MOS (that is, LPN/LVN); requirements include a current, active, valid, and unrestricted LVN/LPN license. Any Soldier who fails to maintain the established LPN/LVN requirements will not perform nursing duties. Soldiers who fail to recertify according to LPN/LVN State licensure requirements will be granted an additional 90 calendar days (for AA) and 180 calendar days (for RC) to complete LPN/LVN licensure renewal. During this time, the unlicensed Soldier will be 100% directly supervised when providing patient care or removed from patient care until license is granted. Soldiers will be deemed MOS qualified during this period. Failure to maintain or renew State licensure following this time period will result in the initiation of administrative action according to governing regulations.

APPENDIX C – DIALYSIS SPECIALTY (68CM3)

1. JOB DESCRIPTION. To provide treatments and care for hemodialysis and peritoneal dialysis patients.

2. MILITARY COURSE. Walter Reed National Military Medical Center, Bethesda, MD.

a. Course length. 20 weeks.

b. Accreditation. None.

c. American Council on Education credit hours. 25 credit hours in addition to the preceding 68C credit hours.

d. School affiliation. None.

e. Degree awarded. None.

f. Eligibility for certification or licensing. None.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision. None.

b. Procedures requiring indirect supervision.

(1) Provides focused hemodialysis patient assessments.

(2) Correctly sets up and maintains the hemodialysis machines and portable reverse osmosis systems.

(3) Sets up and prepares central/femoral lines.

(4) Cannulates a graft or arteriovenous fistula.

(5) Initiates, monitors, and terminates hemodialysis on adult and geriatric patients.

(6) Documents patient status pre-, intra-, and post-dialysis on the appropriate flow sheets.

(7) Provides nursing care to patients with altered fluid volume, nutritional status, and altered tissue perfusion.

(8) Manages patients on hemodialysis with electrolyte imbalance, hyper/hypotension, and decreased cardiac output.

- (9) Correctly uses all equipment in the dialysis unit.

4. RELATED CIVILIAN TRAINING. None.

5. LICENSING/CREDENTIALING.

- a. Army standard/requirement(s) for licensure and/or certification. A current, active, valid, and unrestricted LVN/LPN license.
- b. National and State requirements. None.
- c. Additional requirements after completion of accredited course. None.
- d. Degree requirements. None.
- e. Licensing/certification examination. None.

6. CONTINUING EDUCATION REQUIREMENTS.

- a. Army standard/requirement(s). None.
- b. Maintain civilian license/certification. The dialysis technician must maintain the practical nurse (68C) requirements for licensure. Varies by State National Council Boards of Nursing Licensed Practical/Vocational Nurse requirements.

7. MOS AND ASI REQUIREMENT - LICENSED PRACTICAL/VOCATIONAL NURSE.

Soldiers (AA/RC) must meet and maintain the requirements of the 68C MOS and M3 ASI (that is, LPN/LVN with dialysis specialty). Any Soldier who does not meet the established LPN/LVN requirements will not be permitted to function in any capacity for which they do not qualify. Soldiers who fail to recertify according to LPN/LVN State licensure requirements will be granted an additional 90 calendar days (for AA) and 180 calendar days (for RC) to complete LPN/LVN licensure renewal. Soldiers will be subject to 100% direct supervision or removal from patient care until full licensure is obtained.

APPENDIX D - OPERATING ROOM SPECIALIST (68D)

1. JOB DESCRIPTION. To perform skills in the various roles of a scrub, circulating technician (under the supervision of a registered nurse), and the Sterile Processing Department for direct support of surgical procedures in operating rooms and clinics.

2. MILITARY COURSE. Phase 1: METC, JBSA-Fort Sam Houston, TX; Phase 2: 13 sites.

a. Course length. 21 weeks (Phase 1: 9 weeks; Phase 2: 12 weeks).

b. Accreditation. Accreditation Review Council on Education in Surgical Technology and Surgical Assisting (ARCST/SA) under the auspices of the Commission on Accreditation of Allied Health Education Programs (CAAHEP).

c. American Council on Education credit hours. 4 credit hours.

d. Community College Air Force (CCAF). 15 credit hours

e. School affiliation. None.

f. Degree awarded. None.

g. Eligibility for certification or licensing. Any student enrolled in the course by 1 October 1998 is eligible to sit for the Surgical Technologist National Certifying Examination offered by the Board of Directors of The National Board of Surgical Technology and Surgical Assisting (NBSTA).

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision. Performs the duties of the circulating technician.

b. Procedures requiring indirect supervision.

(1) Applies the principles of sterile technique.

(2) Applies principles of infection control in the operating room and sterile processing department.

(3) Transports patients.

(4) Performs the duties of surgical technologist during surgery.

(5) Assists in positioning and draping patients for surgical procedures.

(6) Prepares and passes sutures, needles, blades, and instruments.

(7) Performs surgical counts.

(8) As a surgical technologist, hands medications, dyes, and hemostatic agents across the sterile field.

- (9) Prepares specimens.
- (10) Applies principles of decontamination, sterilization, and storage of sterile instruments/supplies.
- (11) Processes specialized equipment in Sterile Processing Department.
- (12) Selects appropriate instrument sets.

4. RELATED CIVILIAN TRAINING.

- a. Course length. Vary in length from 1-2 years at different institutions.
- b. Degree awarded. Certificate of completion and Associate Degree in Allied Health
- c. Areas of clinical competency. The majority of accredited programs follow the Association of Surgical Technologists (AST) Core Curriculum (Core Curriculum for Surgical Technology, associated of Surgical Technologists, Inc., 6th Edition, 2011).

5. LICENSING/CREDENTIALING.

- a. Army standards/requirement(s) for licensure and/or certification. None.
- b. National and State requirements. Certification as a certified surgical technologist (CST) is not required in some States, but many civilian hospitals require it for employment.
- c. Additional requirements after completion of accredited course. The only candidates eligible to sit for the certifying examination include graduates of a CAAHEP- accredited course or previous CST.
- d. Degree requirements. None.
- e. Licensing/certification examination. The NBSTA examination is the only examination recognized by the AST.

6. CONTINUING EDUCATION REQUIREMENTS.

- a. Army standard/requirement(s) for licensure and/or certification. None.
- b. Maintain civilian license/certification. 80 continuing education hours in AST categories 1-3 (no more than 10 in category 2).
 - (1) Category 1. Basic sciences, supplies and equipment, surgical procedures, aseptic technique, and patient care.
 - (2) Category 2. Communication, professional development (management, supervision, stress management, ethics, legal, computers, etc.).
 - (3) Category 3. Advanced practice (beyond the introductory or prerequisite level), advance applied science, wound management, or American Medical Association category credits.

APPENDIX E - DENTAL SPECIALIST (68E)

1. JOB DESCRIPTION. To assist the Dental Corps officer in the prevention, examination, and treatment of the diseases of teeth and oral region.

2. MILITARY COURSE. METC, JBSA Fort Sam Houston, TX.

a. Course length. 8 weeks 2 days.

b. Accreditation. None.

c. American Council on Education credit hours. 11 credit hours.

d. School affiliation. None.

e. Degree awarded. None.

f. Eligibility for certification or licensing. There are no licensing requirements for dental assistants. Graduates are eligible to take National Board Examination administered by Dental Assistants National Board (DANB) leading to designation as a certified dental assistant (CDA) and/or are eligible to take the American Medical Technology (AMT) certifying board leading to the designation as a registered dental assistant (RDA).

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision. None.

b. Procedures requiring indirect supervision.

(1) Manages preventive maintenance program for dental equipment, instruments, and assigned vehicles.

(2) Maintains stock level for dental supplies and equipment to support the dental treatment room.

(3) Assists the dentist to maintain patient records and complete clinic reports.

(4) Schedules appointments to make effective use of patient time and maximize productivity.

(5) Prepares the dental operatory prior to treatment.

(6) Performs clinic cleaning, disinfection, sterilization, and uses barrier techniques.

(7) Assist the dentist in all patient care procedures in the clinic and operating room.

(8) Prepares a dental instrument set-up for all diagnostic and treatment procedures including set-ups unique to each specialty care.

(9) Performs four-handed dentistry chair side techniques.

(10) Prepares and/or mixes all restorative and impression material per manufacturer's instructions or dentists' instructions for modification of procedures.

(11) Manages preliminary impressions for diagnostic cast.

(12) Exposes, labels, and saves dental radiographs using the paralleling and panoramic techniques.

(13) Provides patient oral health instruction.

(14) Measures patients' vital signs.

(15) Assists in medical emergency management.

(16) Assists in patient evacuation and in nuclear, biological, and chemical decontamination.

4. RELATED CIVILIAN TRAINING. Dental assistant training program.

a. Course length. 1 year.

b. Degree awarded. Certificate of completion.

c. Areas of clinical competency. Standards for training are set by American Dental Association, Commission on Dental Accreditation, and Accreditation Standards for Dental Assisting Education Programs.

5. LICENSING/CREDENTIALING.

a. Army standards/requirement(s) for licensure and/or certification. None.

b. National and State requirements. None.

c. Additional requirements after completion of accredited course. Applicants who graduate from a DOD dental assisting training program, have at least 2 years' experience in the field, and have a current basic life support card are eligible to take the voluntary DANB examination. Successful completion results in certification as a CDA. A second certifying body, AMT, offers a certifying examination for those with the same qualifications

as the DANB except it requires only 1 years' experience. An AMT- certified dental assistant is designated as an RDA.

d. Degree requirements. None.

e. Licensing/certification examination. Information about DANB certification examination is available by phone 1-800-FOR-DANB or on DANB website <http://www.dentalassisting.com/Default>. AMT certification information is available by phone (847) 823-5169, Fax (847) 823-0458, e-mail amtmail@aol.com, and website <http://www.amt1.com>.

6. CONTINUING EDUCATION REQUIREMENTS.

a. Army standard/requirement(s) for licensure and/or certification. None.

b. Maintain civilian license/certification. 12 continuing education hours per year to maintain voluntary CDA certification. Continuing education hours are encouraged, but not required for an RDA.

APPENDIX F - DENTAL LABORATORY SPECIALTY (68EN5)

1. JOB DESCRIPTION. Upon receiving a laboratory prescription from an Army dentist, fabricate a variety of dental appliances used in the treatment of oral conditions in the restoration of teeth and soft tissue.

2. MILITARY COURSE. METC, JBSA Fort Sam Houston, TX

- a. Course length. 30 weeks (basic phase: 24 weeks; advanced phase: 6 weeks).
- b. Accreditation. Commission on Dental Accreditation (CODA) and Council on Occupational Education (COE).
- c. American Council on Education credit hours. 30 hours; 11 hours of college credit for advanced course can be applied towards associate's degree in dental laboratory technology.
- d. School affiliation. None.
- e. Degree awarded. None.
- f. Eligibility for certification or licensing. Graduates with 5 years' experience and training are eligible to take the National Association of Dental Laboratories (NADL) National Board for Certification examination.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

- a. Procedures requiring direct supervision. None.
- b. Procedures requiring indirect supervision.
 - (1) Completes denture fabrication.
 - (2) Fabricates removable partial dentures.
 - (3) Fabricates fixed prosthetic devices.
 - (4) Repairs dentures.
 - (5) Fabricates orthodontic retainers.
 - (6) Fabricates Metal-Ceramic Restorations and CAD/CAM.

4. RELATED CIVILIAN TRAINING.

- a. Course length. 2 years.

b. Degree awarded. Associate's degree in dental technology.

c. Areas of clinical competency. Standards for training are set by the American Dental Association, Commission on Dental Accreditation, and the Accreditation Standards for Dental Laboratory Technology Education Programs.

5. LICENSING/CREDENTIALING.

a. Army standard/requirement(s) for licensure and/or certification. None.

b. National and State requirements. None.

c. Additional requirements after completion of accredited course. Applicants who graduate from an accredited dental laboratory program and have at least 5 years' training and experience in the field are eligible to take the voluntary NADL National Board for Certification examination.

d. Degree requirements. None.

e. Licensing/certification examination. Voluntary NADL National Board for Certification examination is provided at various locations and at multiple scheduled times. Information may be obtained about the National Board for Certification at phone (800)684-5310 or (850)205-5627, Fax (850)222-0053, email nadl@nadl.org or NADL website <http://www.nadl.org/html/certification.html>.

6. CONTINUING EDUCATION REQUIREMENTS.

a. Army standard/requirement(s). None.

b. Maintain civilian license/certification. In order to maintain voluntary certification, the technician must complete 25 continuing education hours every 2 years.

APPENDIX G - PREVENTIVE DENTISTRY SPECIALTY (68EX2)

1. JOB DESCRIPTION. Provides clinical preventive dentistry services to include oral prophylaxis, fluoride varnish application, pit and fissure sealant application, and patient education.

2. MILITARY COURSE. AMEDDC&S HRCoE, JBSA Fort Sam Houston, TX.

a. Course length. 12 weeks.

b. Accreditation. None.

c. American Council on Education credit hours. None.

d. School affiliation. None.

e. Degree awarded. None.

f. Eligibility for certification or licensing. There is no civilian equivalent for a military preventive dentistry specialty. Since 68E10 is a prerequisite for 68EX2 course, candidates are eligible for the same certifications as graduates of 68E10 course.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision. None.

b. Procedures requiring indirect supervision.

- (1) Records entries into dental record and treatment documentation as a provider.
- (2) Reviews medical history and takes appropriate action prior to care.
- (3) Takes and records vital signs.
- (4) Researches and records implications of pharmaceuticals used by patients.
- (5) Uses dental hygiene instrumentation.
- (6) Customizes oral hygiene instruction and home preventive care to individual patients.
- (7) Capture digital radiographs useful to providing prophylaxis.
- (8) Reviews radiographs for findings relevant to providing preventive care.
- (9) Sharpens instruments.

(10) Conducts intra- and extra-oral inspections, to include recording indexes (for example, periodontal screening and recording) and oral cancer screening.

(11) Applies pit and fissure sealants.

(12) Uses power scaling instruments.

(13) Performs coronal polishing and fluoride varnish application.

(14) Provides oral prophylaxis.

4. RELATED CIVILIAN TRAINING. No civilian equivalent training program is available.

5. LICENSING/CREDENTIALING.

a. Army standard/requirement(s) for licensure and/or certifications. None.

b. National and State requirements. None.

c. Additional requirements after completion of accredited course. No additional requirements follow completion of the course.

d. Degree requirements. None.

e. Licensing/certification examination. None.

6. CONTINUING EDUCATION REQUIREMENTS.

a. Army standard/requirement(s). None.

b. Maintain civilian license/certification. None.

APPENDIX H - PHYSICAL THERAPY SPECIALIST (68F)

1. JOB DESCRIPTION. To perform physical therapy treatment interventions to include the application of therapeutic physical agents, rehabilitation procedures, patient education and injury prevention measures, and limited musculoskeletal evaluations in a safe, ethical manner under the direction and supervision of a licensed physical therapist.

2. MILITARY COURSE. Phase 1: METC, JBSA Fort Sam Houston, TX; Phase 2: 21 sites.

a. Course length. 28 weeks (Phase 1: 18 weeks; Phase 2: 10 weeks).

b. Accreditation. Phase I is accredited by the Commission on Accreditation in Physical Therapy Education (CAPTE) through the Community College of the Air Force (CCAF): 27 Credit Hours

c. American Council on Education credit hours. 8 credit hours.

d. School affiliation. Community College of the Air Force credit hours. 27.

e. Degree awarded. None.

f. Eligibility for certification or licensing. Not eligible until completion of Associate of Applied Science degree.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision. Physical therapy technicians must be supervised by a licensed physical therapist or by a licensed physician in MTFs with no assigned physical therapists.

(1) Performs a baseline assessment on a patient with ankle pathology/injury.

(2) Performs a baseline assessment on a patient with knee pathology/injury.

b. Procedures requiring indirect supervision.

(1) Performs footwear and flexibility screening assessments.

(2) Conducts restorative and preventive back and knee care training.

(3) Administers strength and endurance training programs.

(4) Administers functional programs for treatment of all joint injuries.

- (5) Administers functional programs for neurological and cardiopulmonary patients.
- (6) Provides instruction on use of assistive devices and transfer techniques.
- (7) Performs free flow wound treatment.
- (8) Administers various physical therapy treatments (moist heat pack, cryotherapy, paraffin, pneumatic compression, therapeutic ultrasound, electrical stimulation, transcutaneous electrical nerve stimulation, and biofeedback).
- (9) Administers contrast bath.
- (10) Administers iontophoresis treatment.
- (11) Administers cervical and pelvic traction.
- (12) Performs transverse friction massage.
- (13) Applies upper and lower extremity orthotics.
- (14) Performs residual limb care for upper and lower extremity amputees.
- (15) Administers and/or instructs passive, active assistive, and active range of motion exercises.
- (16) Administers isotonic and isometric exercise programs.
- (17) Administers stretching techniques.

4. RELATED CIVILIAN TRAINING.

- a. Course length. 2 years.
- b. Degree awarded. Associate's degree as a Physical Therapy Assistant (PTA).
- c. Areas of clinical competency. Physical Therapist Assistant Evaluative Criteria, Commission on Accreditation in Physical Therapy Education, American Physical Therapy Association, Adopted November 1, 2000. Effective January 1, 2002.

5. LICENSING/CREDENTIALING.

- a. Army has no standard/requirement(s) for licensure and/or certification.

NOTE: In order to maintain accreditation through CAPTE Army Instructors must have an Associate of Applied Science degree and maintain a state licensure

b. National and State requirements. There are national requirements for licensing or credentialing of civilian PTAs. Licensure is required in all States and requirements vary from State to State.

c. Additional requirements after completion of accredited course. None.

d. Degree requirements. None.

e. Licensing/certification examination. The Federation of State Boards of Physical Therapy governs the national examination. Each State determines the requirements for individuals to be eligible for the examination.

6. CONTINUING EDUCATION REQUIREMENTS.

a. Army standard/requirement(s) for licensure and/or certification. None.

NOTE: Army Instructors will have to follow state requirements. (See below)

b. Maintain civilian license/certification. There is no national requirement. Each State or jurisdiction determines the number of continuing education hours required per certification period. Hours vary from 0 – 3.00 continuing education hours.

b. Must maintain Basic Life Support (BLS) Healthcare Provider recertification requirements.

7. MOS REQUIREMENT.

a. Soldiers (AA/RC) possessing the 68F MOS are required to obtain and maintain certification by the National Heart Association. Certification will be the Basic Life Support for Healthcare Provider and must be recertified every two years.

b. Soldiers who fail to recertify according to American Heart Association guidance will be granted an additional 90 calendar days (for AA) and 180 calendar days (for RC) to obtain BLS certification; Soldiers will be deemed MOS qualified during this period, but must be directly supervised when providing patient care. A Soldier's failure to obtain BLS certification immediately following the respective 90- or 180-day period will result in non-MOS qualified and the initiation of an appropriate personnel action (that is, mandatory reclassification, separation) according to governing regulations.

APPENDIX I – PATIENT ADMINISTRATION SPECIALIST (68G)

1. JOB DESCRIPTION. Primarily responsible for supervising and performing administrative duties in the patient administration division of an Army hospital or other Health Readiness Platforms.

2. MILITARY COURSE. AMEDDC&S, Fort Sam Houston, TX.

- a. Course length. 7 weeks.
- b. Accreditation. None.
- c. American Council on Education credit hours. Not Evaluated.
- d. School affiliation. Community College of the Air Force credit hours. 5.
- e. Degree awarded. None.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

- a. Procedures requiring direct supervision. None.
- b. Procedures requiring indirect supervision.
 - (1) Perform Lifecycle management of all medical records
 - (a) File approved medical documentation in medical records.
 - (b) File a medical record.
 - (c) Retrieve a medical record.
 - (d) Create a medical record.
 - (e) In process a medical record.
 - (f) Out process a medical record.
 - (g) Perform medical record coding.
 - (h) Sequester a medical record.
 - (i) Track medical record.
 - (j) Retire medical records.

- (2) Performs registration functions of all eligible beneficiaries.
 - (a) Verify patient eligibility.
 - (b) Register a patient using electronic health record system(s).
- (3) Perform admission disposition procedures.
 - (a) Perform admission procedures.
 - (b) Activate an admission disposition transfer (ADT).
 - (c) Perform discharge procedures.
 - (d) Generate admission and disposition (A&D) reports.
- (4) Maintain patient accountability during mass casualty (MASCAL) events.
- (5) Maintain patient trust fund
 - (a) Secure patient's personnel effects and clothing.
 - (b) Receive patient funds and valuables for deposit.

4. RELATED CIVILIAN TRAINING. Army standard/requirement(s). None.

5. LICENSING/CREDENTIALING.

- a. Army standard/requirement(s) for licensure and/or certification. None.
- b. Certified Electronic Health Record Specialist (CEHRS). Students attending the 68G Initial Entry Training (IET) course will have an opportunity to sit for the CEHRS. The CEHRS is accredited through the National Healthcareer Association (NHA).

6. CONTINUING EDUCATION REQUIREMENTS. Army standard/requirement(s). None.

APPENDIX J - OPTICAL FABRICATION SPECIALIST (68H)

1. JOB DESCRIPTION. To fabricate and dispense prescription military eyewear using the knowledge of ophthalmic optics, ocular anatomy and physiology, optical laboratory supply and administration, and optical laboratory procedures.

2. MILITARY COURSE. Tri-Service Optician School, Naval Ophthalmic Support Training Activity, Yorktown, VA.

- a. Course length. 24 weeks.
- b. Accreditation Commission on Opticianry Accreditation, and Commission of the Council on Occupational Education.
- c. American Council on Education credit recommendations. 22 semester hours.
- d. School affiliation. J. Sargeant Reynolds Community College, Richmond, VA.
- e. Degree awarded. None.
- f. Eligibility for certification or licensing. Graduates are awarded 26 of the 70 total credit hours required toward completion of an Associate of Applied Science Degree in Opticianry.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision by optometrist or ophthalmologist (skills only taught to familiarization during initial entry training).

- (1) Conducts patient screening procedures.
- (2) Conducts visual field testing.
- (3) Manages medical records.
- (4) Manages the spectacle request transmission system.

b. Procedures requiring indirect supervision.

- (1) Identifies the authorizations for type and amount of military eyewear.
- (2) Identifies the limitations of services.
- (3) Performs inter-pupillary distance measurements.
- (4) Performs segment height measurements.

- (5) Performs vertex depth measurements.
- (6) Orders proper lens styles.
- (7) Performs spectacle fabrication procedures.
- (8) Performs lensometry.
- (9) Adjusts eyeglass frames.
- (10) Dispenses spectacle.

4. RELATED CIVILIAN TRAINING.

- a. Course length. 2-year degree program.
- b. Degree awarded. Associate of Applied Science.
- c. Areas of clinical competency.
 - (1) Correctly preparing spectacle request.
 - (2) Inter-pupillary distance measurements.
 - (3) Segment height measurements.
 - (4) Vertex depth measurements.
 - (5) Ordering proper lens styles.
 - (6) Spectacle fabrication procedures.
 - (7) Lensometry.
 - (8) American National Standards Institute standards.
 - (9) Frame adjustments.
 - (10) Spectacle dispensing.

5. LICENSING/CREDENTIALING.

- a. Army standard/requirement(s) for licensure and/or certification. None.

b. National and State requirements. Many States require licensing; eligibility based on passing American Board of Opticianry (ABO) certification examination. National Contact Lens Examination (NCLE) required for dispensing of contact lenses.

c. Additional requirements after completion of accredited course. Some States require 2 years' dispensing experience before a license is issued after taking the ABO certification exam.

d. Degree requirements. None.

e. Licensing/certification examination. Three levels of certification: basic, advanced, and master's program.

6. CONTINUING EDUCATION REQUIREMENTS.

a. Army standard/requirement(s). None.

b. Maintain civilian license/certification. Varies from 0 hours required to 18 hours (determined by State licensing board). One 3-year renewal period for license renewal (determined by State).

APPENDIX K - MEDICAL LABORATORY SPECIALIST (68K)

1. JOB DESCRIPTION. The medical laboratory specialist performs blood banking procedures and elementary and advanced examinations of biological and environmental specimens to aid in the diagnosis, treatment, and prevention of disease and other medical disorders.

2. MILITARY COURSE. Phase 1: METC, JBSA Fort Sam Houston, TX;
Phase 2: 19 Sites

- a. Course length. 52 weeks (Phase 1: 26 weeks; Phase 2: 26 weeks).
- b. Accreditation. Received national accreditation from the National Accrediting Agency for Clinical Laboratory Sciences (NAACLS).
- c. American Council on Education credit hours. 36 credit hours.
- d. School affiliation. George Washington University, Washington, D.C.
- e. Degree awarded. None. However, the student may be eligible to apply for an associate's degree from George Washington University after completing an additional 15 semester hours in English; math; biology or chemistry; humanities; and a social science.
- f. Eligibility for certification or licensing. Graduates are eligible to take the American Society for Clinical Pathology (ASCP) Board of Certification (BOC) Examination; the American Medical Technologists (AMT) Certification Examination; and the American Association of Bio analysts (AAB) Board of Registry (ABOR) Certification.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

- a. Procedures requiring direct supervision. None.
- b. Procedures requiring indirect supervision.
 - (1) Collects venous and capillary blood specimens.
 - (2) Performs point of care testing, quality control, and reporting for the following types of specimens:
 - (a) Hematology.
 - (b) Chemistry.
 - (c) Microbiology.
 - (d) Blood Banking.

NOTE: Medical laboratory specialists who provide direct patient care must have been instructed and demonstrated competencies related to performance of all tasks (per Department of Health and Human Services, Health Care Financing Administration, Clinical Laboratory Improvement Amendments of 1998 and DOD Clinical Laboratory Improvement Program).

4. RELATED CIVILIAN TRAINING.

- a. Course length. 18 months to 2 years.
- b. Degree awarded. Associate's degree in applied science or certificate through a certification program.
- c. Areas of clinical competency. See survey results of the Associate Member Section of the American Society for Clinical Pathology (ASCP AMS). Clinical competencies encompass different subtests within the area of Medical Laboratory Science: Blood Banking, Urinalysis and Other Body Fluids, Chemistry, Hematology, Immunology, Microbiology and Laboratory Operations.

5. LICENSING/CREDENTIALING.

- a. Army standard/requirement(s) for licensure and/or certification. None.
- b. National and State requirements. Passing the ASCP, AMT, or AAB examination are all national certifications. Some States require additional State certification/licensure. There are three credentialing agencies: the American Society for Clinical Pathology (ASCP) Board of Certification (BOC), the American Medical Technologists (AMT), and the American Association of Bio analysts (AAB) Board of Registry (ABOR). The ASCP, AMT and AAB all offer certification as a medical laboratory technician.
- c. Additional requirements after completion of accredited course. To be eligible for this examination category, an applicant must satisfy the requirements of at least one of the following certification agencies:
 - (1) Routes for national certification under American Society for Clinical Pathology (ASCP) are located at: <https://www.ascp.org/content/board-of-certification/getcertified#tabs-eligibility>.
 - (2) Routes for national certification under American Medical Technologists (AMT) are located at: <http://www.americanmedtech.org/GetCertified/MLTEligibility.aspx>.
 - (3) Routes for national certification under American Association of Bio analysts (AAB) are located at: <http://www.aab.org/aab/MLT.asp>.
- d. Degree requirements. As listed above.

e. Licensing/certification examination. ASCP Board of Certification examination, AMT certification or the AAB Board of Certification as described above.

6. CONTINUING EDUCATION REQUIREMENTS.

a. Army standard/requirement. None.

b. Maintain civilian license/certification. In order to maintain voluntary certification, the technician must complete the continuing education requirements for the respective certification agency:

(1) For ASCP MLT certified individuals, the Credential Maintenance Program (CMP) is required for all individuals who became newly certified on or after 1 January 2004. Voluntary participation in the CMP is encouraged for individuals certified prior to 1 January 2004.

(2) For AMT certified individuals, the Certification Continuation Program (CCP) requires AMT certified members to document activities supporting continuation of AMT certification every 3 years.

(3) ABOR does not have a CEU requirement in order to maintain the certification; however, they do offer a continuing education program.

APPENDIX L – CYTOTECHNOLOGY SPECIALITY (68KM2)

1. JOB DESCRIPTION. To perform microscopic screening and interpretation of positive gynecological and nongynecological diagnostic specimens while rendering cytologic diagnoses for negative gynecological specimens. Provides aid to the cytopathologist and pathologist in the diagnosis, treatment, and prevention of disease.

2. MILITARY COURSE. AMEDDC&S HRCoE, JBSA Fort Sam Houston, TX.

a. Course length. 50 weeks (Didactic and Clinical included).

b. Accreditation. Received national accreditation from the Commission on Accreditation of Allied Health Education Programs (CAAHEP).

c. American Council on Education credit hours. No credits awarded.

d. School affiliation. The George Washington University, Washington, D.C.

e. Degree awarded. Baccalaureate Degree.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision. None.

b. Procedures requiring indirect supervision.

(1) Performs cytologic screening and interpretation of specimen samples to include specimen adequacy for the following areas:

a. Gynecological.

b. Nongynecological.

c. Surgical/Histological

(2) Provide assistance during rapid onsite assessment and Fine Needle Aspiration (FNA) procedures.

(3) Correlate cellular abnormalities and patient history while providing accurate prediagnosics interpretations.

(4) Utilize basic laboratory skill sets in the processing, staining and microscopic evaluation of gynecological and nongynecological specimens.

(5) The cytotechnologists will be able to understand quality control and quality assurance requirements within the Anatomical Pathology department. Implement corrective actions and control measures as needed.

(6) Effectively utilize Laboratory Information Systems (LIS) including but not limited to viewing patient history, entering results and signing out cases.

(7) The graduate will be able to explain the theory, principles and indications of the following ancillary techniques:

(a) Flow cytometry.

(b) Molecular signal detection.

(c) Molecular diagnostic oncology.

(d) Computer-based image-analysis and its applications in ancillary tests applied to diagnostic cytopathology.

(8) The graduate will be able to explain the theory, principles, indications, technical aspects, and troubleshooting of:

(a) Human Papillomavirus (HPV) Deoxyribonucleic Acid (DNA) testing.

(b) Immunocyto-histochemistry (IHC/ICC).

(d) Fluorescence and Chromogenic in-situ hybridization (FISH/CISH).

(e) Standard laboratory digital pathology equipment and procedures, such as, but not limited to digital cameras and photography, digital whole slide imaging scanners, image management, web-screen sharing, and slide viewing.

(9) Understand the use of evidenced-based medicine and the requirements for professional development/certification maintenance.

NOTE: Cytotechnologists who provide direct patient care must have been instructed and demonstrated competencies related to performance of all tasks (per Department of Health and Human Services, Health Care Financing Administration, Clinical Laboratory Improvement Amendments of 1998 and DOD Clinical Laboratory Improvement Program).

4. RELATED CIVILIAN TRAINING.

a. Course length. 2 years to 4 years.

b. Degree awarded. Baccalaureate Degree or Master Degree in Cytotechnology.

c. Areas of clinical competency. Cytotechnologists must take and pass the federally mandated annual Pap smear proficiency examination with a score of 90% or higher per the Centers for Medicare and Medicaid Services and 42 CFR 493.855 Standard; Cytology; gynecologic examinations. <https://www.cms.gov/Regulations-and-guidance/Legislation/CLIA/CytologyProficiencyTesting.html>

5. LICENSING/CREDENTIALING.

a. Army standard/requirement(s) for licensure and/or certification. Maintain ASCP Cytotechnologist (CT) certification in accordance with paragraph 6b of this appendix.

b. National and State requirements. Passing the ASCP examination are both national requirements and some States require additional State certification and/or licensure. There is only one credentialing agency authorized for the United States: the American Society for Clinical Pathologists (ASCP) Board of Certification (BOC). The ASCP offers certification as a Cytotechnology Specialist (SCT) while the International Academy of Cytology (IAC) offers certification as a CT for international screening.

c. Additional requirements after completion of accredited course. To be eligible for this examination category, an applicant must satisfy the requirements of the following route: Baccalaureate degree or higher from a regionally accredited college/university and successful completion of a CAAHEP accredited Cytotechnology program within the last five years.

d. Degree requirements. Baccalaureate degree from a CAAHEP accredited Program, however, the student must meet the following prerequisites: Candidates must possess an Associate Degree or must have completed 60 semester hours (SH) with a grade of "C" or higher from an accredited college, university, or equivalent training. Academic preparatory work must include a minimum of 20 SH in biological sciences, 3 SH of college algebra, 8 SH in chemistry, 6 SH in Humanities, 12 SH of Social science and 6 SH of English with composition.

e. Licensing/certification examination. ASCP Board of Certification examination CT as described above.

6. CONTINUING EDUCATION REQUIREMENTS.

a. Army standard/requirement. Maintenance of ASCP Cytotechnology certification as described in 6b and passing score of 90% or higher on the annual Pap smear Proficiency examination in order to diagnosis patient samples.

b. Maintain civilian license/certification. Continuing Medical Education (CME) or Continuing Education (CE) credits are voluntary for individuals certified before 2004. For individuals certified after 2004, total number of CME/CE credits total 36 with the following requirements: 1 point in laboratory or patient safety and 2 points in the area of Cytology and remaining points in the areas of specialty, management, education, or other related laboratory areas of interest.

APPENDIX M – OCCUPATIONAL THERAPY SPECIALIST (68L)

1. JOB DESCRIPTION. To provide competent patient assessment, treatment, documentation, health promotion, and injury prevention in both clinical and field environments under the supervision of a qualified Registered Occupational Therapist (OTR).

2. MILITARY COURSE. Phase 1: METC, JBSA Fort Sam Houston, TX.
Phase 2: 18 sites.

- a. Course length. 33 weeks (Phase 1: 18 weeks, Phase 2: 16 weeks).
- b. Accreditation. Accreditation Council for Occupational Therapy Education (ACOTE) of the American Occupational Therapy Association.
- c. American Council on Education credit hours. 66 hours.
- d. School affiliation. University of the Incarnate Word.
- e. Degree awarded. Associate of Science in Health Science.
- f. Eligibility for certification and licensing. Graduates are eligible to take the national certification examination offered by the National Board for Certification in Occupational Therapy. After successful completion of this examination, graduates have the option to become licensed certified Occupational Therapy Assistants (OTAs) depending on individual State requirements.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision. Non-Certified OTAs require direct supervision during their practice. Certified OTAs required indirect supervision after 1 year of supervised practice. All OTAs can operate under the license of the Occupational Therapist Registered (OTR).

b. Procedures requiring indirect supervision for Certified OTAs.

(1) Performs activities of daily living assessment and assists in the development of a treatment plan.

(2) Conducts ongoing assessments of the client's progress and collaborates with the OTR to modify the treatment plan accordingly.

(3) Provides holistic therapeutic rehabilitation based on the client's occupational performance within the context of the client's deficits.

(4) Assesses functionality of the upper extremities and the client's overall performance.

(5) Performs standardized cognitive and perceptual testing.

(6) Performs wound management.

(7) Applies physical agent modalities to the upper extremity.

(8) Conducts activity analysis.

(9) Conducts upper extremity rehabilitation.

(10) Creates static and dynamic orthosis to assist, maintain, or regain functional or normal range of motion.

(11) Provides activity and occupation-based treatment modalities and interventions for psychosocial treatment.

(12) Performs ergonomic evaluations and provides recommendations to adapt and enhance the occupational environment.

(13) Performs upper extremity sensory and neuromuscular testing.

4. RELATED CIVILIAN TRAINING.

a. Course length. 2 years.

b. Degree awarded. Associate's degree.

c. Areas of clinical competency. Same as military course as prescribed by American Occupational Therapy Association and National Board for Certification in Occupational Therapy (NBCOT).

5. LICENSING/CREDENTIALING.

a. Army standard/requirement(s) for licensure and/or certification.

b. National and State requirements. None.

c. Degree requirements. Requirements vary based on the sponsoring institution; however, all programs must meet national standards established by the ACOTE.

d. Certification examination/Licensure. The national certification examination is offered throughout the year. However, if an individual fails the national certification

examination, he or she may re-apply immediately to re-test but must wait a period of 30 days before re-taking the exam. Licensure requirements vary from State to State.

e. Future requirements. All educational programs are to be in compliance with the new "Standards" established by ACOTE and effective 31 July 2013.

6. CONTINUING EDUCATION REQUIREMENTS.

a. There is no Army requirement for licensure or certification.

b. Hours required to maintain license/certification. To maintain national certification, an individual must remain in good standing (have no pending legal actions/malpractice issues, etc.), complete 36 approved professional development units (PDUs) as required by NBCOT, and must pay a renewal fee every 3 years. State licensure requirements vary, but most require 12-15 credit hours per year of continuing education.

APPENDIX N - NUTRITION CARE SPECIALIST (68M)

1. JOB DESCRIPTION. To perform basic clinical dietetics functions in the dietary management and treatment of patients in nutrition clinics, medical treatment facilities, field operational combat support hospitals, and health promotion and wellness clinics.

2. MILITARY COURSE. METC, JBSA Fort Sam Houston, TX.

a. Course length. Soldiers first attend and qualify in the 92G MOS (8 weeks), then attend the 7-week initial entry training course.

b. Accreditation. Association of Nutrition and Food Science Professionals.

c. American Council on Education credit hours. 4 Semester hours.

d. School affiliation. Community College of the Air Force. 7 hours.

e. Degree awarded. None.

f. Eligibility for certification or licensing. None.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision.

- (1) Provides basic dietary instruction to a group or individual.
- (2) Provides nutritional intervention and management for a specified diet.
- (3) Provides and documents drug nutrient interaction counseling.
- (4) Interviews patient and obtains dietary history.
- (5) Processes clinical dietetic documents.
- (6) Prepares nourishment, forced fluids, and enteral formulas.
- (7) Processes diet additions, changes, and deletions IAW Physicians' orders.
- (8) Inputs and retrieves patient information.
- (9) Assists in community programs and health and wellness promotion.

b. Procedures requiring indirect supervision.

- (1) Checks patient trays for accuracy.

- (2) Conducts nutritional screening
- (3) Assembles patient trays in accordance with prescribed diet.
- (4) Delivers patient trays.

4. RELATED CIVILIAN TRAINING. Certified Dietary Manager, Certified Food Protection Professional (CDM-CFFP), ServSafe Food Protection Manager, Dietetic Technician, Registered (DTR)

5. LICENSING/CREDENTIALING. Optional: Certified Dietary Manager, Certified Food Protection Professional (CDM-CFFP), ServSafe Food Protection Manager, Dietetic Technician, Registered (DTR)

6. CONTINUING EDUCATION REQUIREMENTS.

- a. Army standard/requirement(s). None.
- b. Maintain license/certification. N/A.

APPENDIX O - CARDIOVASCULAR SPECIALTY (68N)

1. JOB DESCRIPTION. To perform noninvasive and invasive procedures under supervision of a cardiologist for a broad range of patients in a cardiovascular setting, assist physicians in performing diagnostic and therapeutic cardiac procedures, and operate and maintain sophisticated medical equipment.

2. MILITARY COURSE. Phase 1: AMEDDC&S HRCoE, JBSA Fort Sam Houston, TX; Phase 2: Four Sites.

- a. Course length. 57 weeks (Phase 1: 21 weeks; Phase 2: 36 weeks).
- b. Accreditation. Invasive Cardiovascular Technology CAAHEP, Joint Review Committee on Education in Cardiovascular Technology (JRC-CVT) until 2024
- c. American Council on Education credit hours. 57 credit hours.
- d. School affiliation. None.
- e. Degree awarded. None.
- f. Eligibility for certification or licensing. Graduates are eligible to take national Registry Examination for Registered Cardiovascular Invasive Specialty (RCIS) and Registered Cardiac Sonographer.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

- a. Assist the cardiologist in these high level diagnostic procedures
 - (1) Right heart catheterization.
 - (2) Left heart catheterization.
 - (3) Percutaneous transluminal coronary angioplasty.
 - (4) Right ventricular biopsy.
 - (5) Coronary artery stenting.
 - (6) Directional coronary atherectomy.
 - (7) Rotoblator procedure.
 - (8) Temporary and permanent pacemaker implantation.
 - (9) Implantable cardio defibrillator.

- (10) Pulmonary angiography.
- (11) Aortic/peripheral angiograms.
- (12) Electrophysiology study.
- (13) Angiojet procedure.
- (14) Valvuloplasty procedure.
- (15) Doppler flow wire procedure.
- (16) Intravascular ultrasound procedure.
- (17) Intraaortic balloon pump insertion and management.
- (18) Pericardiocentesis procedure.
- (19) Pediatric heart catheterization.
- (20) Transesophageal echocardiogram.
- b. Procedures requiring indirect supervision.
 - (1) 12-lead electrocardiogram.
 - (2) Treadmill stress testing.
 - (3) Holter monitor.
 - (4) Pacemaker check.
 - (5) Transthoracic echocardiogram.

4. RELATED CIVILIAN TRAINING.

- a. Course length. Invasive cardiovascular technician – 2 years, noninvasive cardiovascular technician – 2 years.
- b. Degree awarded. Associate's degree in cardiovascular technology.
- c. Areas of clinical competency. Entry level, invasive, echocardiographic, and noninvasive competencies are described in The Scope of Practice for the Registered Cardiovascular Invasive Specialist (RCIS) By The Society of Invasive Cardiovascular Professionals (SICP) Rev 5/2010 and the Scope of Practice and Clinical Standards for the Diagnostic Medical Sonographer by the Society of Medical Sonography, 2015.

5. LICENSING/CREDENTIALING.

- a. Army standard/requirement(s) for licensure and/or certification. None.
- b. National and State requirements. Currently, several States mandate credentialing to obtain reimbursement of vascular ultrasound procedures. Some States have legislative acts that require personnel to hold the Registered Cardiovascular Invasive Specialist (RCIS) or Registered Cardiac Sonographer (RCS) credential to work as a cardiovascular technologist.
- c. Additional requirements after completion of accredited course. Applicants who graduate from a programmatically accredited program in invasive cardiovascular technology are eligible to sit for the national registry exam. All other applicants must meet the following qualifications:
 - (1) Graduated of an associate, or baccalaureate academic program in health science and,
 - (2) Have 1 year full-time work experience in invasive cardiovascular technology and,
 - (3) Have 600 cardiac diagnostic/interventional procedures in their career which is defined as work experience and/or clinical experience gained during a formal educational program.
- d. Degree requirements. Degree requirements throughout the United States vary but most employers require the RCIS credential within 1 or 2 years of hiring and graduation from a CAAHEP-accredited cardiovascular program.
- e. Licensing/certification examination. Currently, Cardiovascular Credentialing International (CCI) is the sole agency responsible for credentialing invasive cardiovascular technologists. The national registry examination is offered throughout the year and is composed of 150-200 multiple choice questions and has a total testing time of 3 hours. CCI awards the RCIS credential to those individuals who score at least 70 percent. The sonographer certification can be obtain from two entities: The Registered Cardiac Sonographer (RCS) from CCI, or the Registered Diagnostic Cardiac Sonographer (RDCS) through the American Registry for Diagnostic Medical Sonography (ARDMS).

6. CONTINUING EDUCATION REQUIREMENTS.

- a. Army standard/requirement(s) for licensure and/or certification. None.
- b. Maintain civilian license/certification. Personnel holding the RCIS or RCS or credential are required to accrue 36 contact hours of continuing education over a 3 calendar-year period. Of the 36 contact hours needed, 16 must be cardiovascular specific.

APPENDIX P - RADIOLOGY SPECIALIST (68P)

1. JOB DESCRIPTION. To perform medical radiography and imaging procedures and related patient care duties.

2. MILITARY COURSE. Phase 1: METC, JBSA Fort Sam Houston, TX;
Phase 2: 17 sites.

a. Course length. 46 weeks (Phase 1: 24 weeks; Phase 2: 22 weeks).

b. Accreditation. Joint Review Committee on Education in Radiologic Technology (JRCERT) evaluated the resident course and its participating clinical sites and granted full accreditation in (AWAITING TERM OF ACREDITATION, VISIT DONE MAY 2016).

c. American Council on Education – 3 credit hours; Community College of the Air Force – 41 credit hours.

d. School affiliation. None.

e. Degree awarded. None.

f. Eligibility for certification or licensing. Graduates are eligible to take the American Registry of Radiologic Technologist (ARRT) national examination upon completion of their Associates degree within three years of graduation from the program. (New requirement from accrediting body effective January 2015).

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision (staff technologist present to observe and correct as needed the performance of the student).

(1) Cases in the Surgical Suites or Operating Room.

(2) Mobile or portable procedures.

(3) Trauma cases.

(4) Inpatients, critically ill, patients on gurneys and patients in wheel chairs.

(5) Pediatric cases 13 years or younger

(6) Pregnant patients.

(7) Repeating unsatisfactory images.

b. Procedures requiring indirect supervision (staff technologist will be available).

- (1) Assumes responsibility for assigned area and reports equipment malfunction.
- (2) Gathers equipment to perform radiographic procedures.
- (3) Responsible for control of inventory and supplies for the assigned area.

4. RELATED CIVILIAN TRAINING.

- a. Course length. 2-5 year college based programs.
- b. Degree awarded. None.
- c. Areas of clinical competency. The ARRT established the scope of practice as outlined in Primary Certification and Registration Didactic and Clinical Competency Requirements, dated January 2016. The American Registry of Radiologic Technologists, 1255 Northland Drive, St. Paul, MN 55120-1155. www.arrt.org

5. LICENSING/CREDENTIALING.

- a. Army standard/requirement(s) for licensure and/or certification. None.
- b. National and State requirements. Most States require licensure for individuals operating x-ray-producing equipment while performing radiographic procedures. The ARRT administers national credentialing for the radiographer and has reciprocal agreements with most States for recognition of credentials.
- c. Completion of accredited course. Graduates are eligible to take the ARRT national examination upon completion of their Associates degree must be within three years of graduating the program.
- d. Degree requirements. Practicing radiographers must hold at least an associate's degree or equivalent to work as a radiographer in a full capacity.
- e. Licensing/certification examination. States conduct their own licensure examinations or recognize the national registry examination as meeting their requirements.

6. CONTINUING EDUCATION REQUIREMENTS.

- a. Army standard/requirement(s). None.
- b. Maintain civilian license/certification. Registrants with the ARRT are required to complete 24 continuing education hours (24 every 2 years) to maintain registry. States have their own individual requirements for continuing education credits toward license maintenance.

APPENDIX Q – NUCLEAR MEDICINE SPECIALTY (68PM5)

1. JOB DESCRIPTION. To perform nuclear medicine imaging procedures and related patient care duties at fixed MTFs.

2. MILITARY COURSE. Phase 1: METC, JBSA-Fort Sam Houston, TX;
Phase 2: 5 sites.

a. Course length. 52 weeks (Phase 1: 20 weeks; Phase 2: 32 weeks).

b. Accreditation. Community College of the Air Force and Council on Occupation Education (COE).

c. American Council on Education credit hours. 0.

d. School affiliation. Community College of the Air Force credit hours. 66.

e. Degree awarded. None.

f. Eligibility for certification or licensing. The American Registry of Radiologic Technologist (ARRT) stipulates that the technologist must have successfully completed a Nuclear Medicine Technology educational program that is accredited by a mechanism acceptable to the ARRT and must have earned an academic degree before becoming certified and registered. The Nuclear Medicine Technology Certification Board (NMTCB) only allows graduates of approved nuclear medicine technology programs are eligible to take NMTCB Certification Examination.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision.

(1) Performs central nervous system, pulmonary, cardiac, gastrointestinal, urinary tract, and bone imaging studies.

(2) Performs tumor and inflammatory process imaging studies.

(3) Performs special imaging studies.

(4) Performs quality control procedures on nuclear imaging devices.

(5) Performs endocrine diagnostic and imaging procedures.

(6) Performs single photon emission computed tomography imaging.

(7) Performs the elution of a radionuclide generator system.

- (8) Prepares radiopharmaceutical kits.
- (9) Performs radiopharmaceutical dose administration.
- (10) Performs thyroid therapy procedures.
- (11) Performs staff bioassay procedures.
- (12) Performs cell-tagging procedures.
- (13) Performs blood component evaluations.
- (14) Performs Schilling's test.
- (15) Performs glomerular filtration rate determinations.
- b. Procedures requiring indirect supervision.
 - (1) Performs computer application procedures.
 - (2) Performs dose calibrator quality control.
 - (3) Prepares a personnel dosimetry program.
 - (4) Prepares a renewal and/or amendment for a radioactive material license or permit.
 - (5) Determines the shielding requirements for the radionuclide generator system.
 - (6) Prepares and follows approved protocol for handling radiation accidents and casualties at the MTF.
 - (7) Prepares a protocol for handling misadministration at the MTF.
 - (8) Performs area monitoring procedures to mitigated radiation contaminated areas
 - (9) Schedules patient

4. RELATED CIVILIAN TRAINING. Nuclear medicine technology education will transition to 2-4 year college based programs awarding associates and/or bachelor degrees.

- a. Course length. 2-4 years.

b. Degree awarded. Associate's degree and/or Bachelor's Degree, nuclear medicine technologies.

c. Areas of clinical competency.

(1) Patient care and patient record keeping.

(2) Radiation safety techniques.

(3) Equipment quality control.

(4) Radiopharmacy.

(5) Performance of imaging studies.

5. LICENSING/CREDENTIALING.

a. Army standard/requirement(s) for licensure and/or certification. None.

b. National and State requirements. Must have successfully completed an accredited program and pass the national examination by the ARRT and (NMTCB).

c. Additional requirements after completion of accredited course. ARRT stipulate that candidates must have earned an academic degree before attaining certification and registration in Nuclear Medicine Technology.

d. Degree requirements. None.

e. Licensing/certification examination. ARRT and NMTCB.

6. CONTINUING EDUCATION REQUIREMENTS.

a. Army standard/requirement(s). None.

b. Maintain civilian license/certification. 24 category A or A+ credits of continuing education evaluated by a recognized by a Recognized Continuing Education Mechanism (RCEEM) or RCEEM + every 2 years.

APPENDIX R - PHARMACY SPECIALIST (68Q)

1. JOB DESCRIPTION. To prepare, control, and issue pharmaceuticals to maintain health and aid in the diagnosis, treatment, and prevention of disease.

2. MILITARY COURSE. METC, JBSA-Fort Sam Houston, TX.

- a. Course length. 22 weeks, 2 days.
- b. Accreditation. American Society of Health System Pharmacists (ASHP).
- c. American Council on Education (ACE). 4 credit hours
- d. School affiliation. Community College of the Air Force. 35.
- e. Degree awarded. None.
- f. Eligibility for certification or licensing. Graduates are eligible to take the National Pharmacy Technician Certification Exam (PTCE) and qualify for the designation, Certified Pharmacy Technician (CPhT).

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision. A pharmacy NCO, pharmacist, or physician on orders as the pharmacy officer will supervise initial entry pharmacy technicians in the performance of the following procedures:

(1) Receives and reviews prescriptions for the full range of pharmaceuticals and patient ages. Obtains the pharmaceutical, prepares the label, and sets up the prescription for review.

(2) Interprets and fills individual orders (prescriptions, unit dose, or intravenous orders) to include controlled substances. Inputs medication orders into the hospital computer system to ensure no overlapping orders or prepares a label using manual methods. Technician selects type of medication, dosage form, strength, and dosage of medications.

(3) Screens individual prescriptions and refills prior to preparing labels or filling for compliance with Federal laws, Army regulations, DOD policies, and established pharmacy policies governing substitutions, quantity, dispensing requirements, and restrictions.

(4) Receives and interprets orders and prepares intravenous additives and other sterile product preparations. Uses aseptic technique to prepare sterile products and intravenous solutions.

b. Procedures requiring indirect supervision.

(1) Inventories, requisitions, receives, and verifies quantity, quality, potency date, and storage of pharmaceutical items including controlled substances, expired, and recalled medications.

(2) Stores pharmaceuticals in accordance with manufacturer's specifications to ensure preservation of all medications that may be subject to change in their chemical properties.

(3) Receives, screens, and fills bulk drug orders for controlled and non-controlled drugs to include accountability procedures.

(4) Calculates work units.

(5) Performs specific compounding when needed for all substances for which formula cards or specific instructions are available.

c. Clinical competency (advanced). Listed below are procedures that require minimal supervision and are assigned upon completion (as appropriate) of a competency assessment program. Procedures and conduct of such programs are in accordance with local policies and guidelines.

(1) Conducts ward and clinic inspections.

(2) Reviews profile for allergies, drug-drug or drug-food interactions, and duplicate orders. Coordinates with prescriber or pharmacist to clarify issues or problems with dosing, interactions, or instructions.

(3) Prepares full range of sterile products and performs pharmaceutical calculations for a wide variety of preparations.

(4) Dispenses new and refill prescriptions that have been evaluated for correctness and accuracy by a pharmacist.

4. RELATED CIVILIAN TRAINING. Pharmacy technician training programs in the civilian sector are not standardized. The scope of practice for the pharmacy technician in a military facility is much broader than the pharmacy technician counterpart in civilian practice where the focus (in the civilian sector) is support of the pharmacist.

a. Course length. Courses range from 4 months to 2 years.

b. Degree awarded. Some colleges offer an associate's degree upon completion, but most programs offer a certificate or training diploma.

c. Areas of clinical competency. No national standards established.

5. LICENSING/CREDENTIALING.

- a. Army standard/requirement(s) for licensure and/or certification. None.
- b. National and State requirements. Currently there is no national requirement for certification. Several States have implemented requirements for national certification or registration with their respective State Boards of Pharmacy.
- c. Additional requirements after completion of accredited course. None.
- d. Degree requirements. None.
- e. Licensing/certification examination. The PTCE is administered at more than 350 Pearson Professional and military test centers nationwide throughout the year. Test dates, locations, and more information is available at <http://www.ptcb.org>.

6. CONTINUING EDUCATION REQUIREMENTS.

- a. Army standard/requirement(s). None.
- b. Maintain civilian license/certification. Recertification must be conducted every two years in order to maintain the Pharmacy Technician National Certification. A requirement of 20 continuing education (CE) hours (must include 1CE each in Pharmacy Law and Medication Safety) must be provided with the application for renewal. Submission of the application and supporting documents must be submitted prior to the expiration date

APPENDIX S - EAR, NOSE, AND THROAT (ENT) SPECIALIST (68U)

1. JOB DESCRIPTION. To assist the otolaryngologist and audiologist in examination and treatment procedures and support installation hearing programs as certified ENT Technicians.

2. MILITARY COURSE. Phase 1: AMEDDC&S HRCoE, JBSA Fort Sam Houston, TX; Phase 2: Nine sites.

a. Course length. 14 weeks (Phase 1: 7 weeks; Phase 2: 7 weeks).

b. Accreditation. Certification by the Council for Accreditation in Occupational Hearing Conservation (CAOHC) is maintained through review of course curriculum for each iteration.

c. American Council on Education credit hours. 2 credit hours. 4 vocational hours.

d. School affiliation. Community College of the Air Force. 6.

e. Degree awarded. None.

f. Eligibility for certification or licensing. After completion of Phase 1, students are certified as DOD Hearing Technicians by the Army Hearing Program, Army Public Health Center (P).

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision by an otolaryngologist. Serves as surgical technician during ear, nose, and throat (ENT) surgical procedures.

b. Procedures requiring indirect supervision by an audiologist.

c. Validates functioning of the manual audiometer.

(1) Completes pure tone and speech diagnostic audiometry.

(2) Conducts acoustic emittance testing.

(3) Fits preformed earplugs.

(4) Validates functioning of the Defense Occupational Environmental Health Readiness System–Hearing Conservation.

(5) Conducts monitoring audiometry.

(6) Produces a hearing aid ear mold impression.

(7) Repairs minor hearing aid deficiencies.

- (8) Performs otoacoustic emissions screening.
- d. Procedures requiring indirect supervision by an otolaryngologist.
 - (1) Cleans examination/operating rooms and equipment.
 - (2) Prepares clinic operating room for surgical procedures.
 - (3) Prepares examination room for multiple minor surgical procedures.
 - (4) Inserts anterior nasal packing.
 - (5) Performs cerumenectomy, ear irrigation, or ear foreign body removal.
 - (6) Maintains tracheostomies.
 - (7) Maintains endoscopic equipment.
 - (8) Applies mastoid dressing.
 - (9) Provides follow-up care for removal of stitches, staples, and nasal splints.

4. RELATED CIVILIAN TRAINING. Council for Accreditation in Occupational Hearing Conservation (CAOHC).

5. LICENSING/CREDENTIALING.

- a. Army standard/requirement(s) for licensure and/or certifications. None.
- b. National and State requirements. None.
- c. Degree requirements. None.
- d. Licensing/certification examination. 68U students have the opportunity to challenge the civilian certification, CAOHC, prior to MOS completion.

6. CONTINUING EDUCATION REQUIREMENTS.

- a. Army standard/requirement(s) for licensure and/or certification. None.
- b. Maintain civilian license/certification. Maintain certification as DoD Hearing Technicians by the Army Public Health Center (P).

APPENDIX T - RESPIRATORY THERAPIST (68V)

1. JOB DESCRIPTION. Assists physicians in the clinical health management of patients that require respiratory therapy, mechanical ventilation, or pulmonary function testing. Provides respiratory therapy as a member of a Critical Care Air Transport Team in the transport of critically ill patients. Assists in the clinical health management of patients with sleep apnea and other sleep disorders. Assists Pulmonologists with invasive procedures and surgical biopsies.

2. MILITARY COURSE. Phase 1: METC, JBSA Fort Sam Houston, TX; Phase 2: San Antonio Military Medical Center (SAMMC).

- a. Course length. 44-weeks (Phase 1: 28-weeks; Phase 2: 16-weeks).
- b. Accreditation. Commission of the Accreditation of Respiratory Care (CoARC).
- c. American Council on Education credit hours. 37 credit hours.
- d. School affiliation. The University of the Incarnate Word.
- e. Degree awarded. Associate Degree in an Applied Science in Respiratory Care. (60 semester hours towards an Associate Degree, 12 semester hours towards a Bachelor Degree, a total of 72 semester hours at graduation.)
- f. Eligibility for certification or licensing. All graduates are eligible to challenge the Therapist Multiple Choice (TMC) examination offered by The National Board for Respiratory Care (NBRC). The TMC awards the CRT credential and allows for the possibility to be RRT eligible if test score is above a certain threshold. The TMC examination is paid for by the 68V schoolhouse because the exam pass rates are used as a metric for the CoARC accreditation.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

- a. Procedures requiring direct supervision. Assists pulmonologist in performing fiberoptic bronchoscopy.
- b. Procedures requiring indirect supervision. (The following list is not all inclusive, refer to the latest Critical Task List.)
 - (1) Administers aerosol therapy.
 - (2) Administers respiratory care drugs.

- (3) Administers oxygen therapy.
- (4) Maintains an airway.
- (5) Conducts high frequency ventilation on a patient.
- (6) Reacts to respiratory emergency.
- (7) Performs postural drainage and other physiotherapy techniques.
- (8) Performs post-surgical exercises.
- (9) Performs arterial puncture and blood gas analysis.
- (10) Conducts initial respiratory assessment.
- (11) Conducts various forms of invasive and non-invasive mechanical ventilation.
- (12) Performs pulmonary function testing and bronchoprovocation testing.
- (13) Performs nasotracheal suctioning.
- (14) Weans a patient from ventilatory support.
- (15) Changes-out breathing circuit.
- (16) Transports a patient on ventilatory support.
- (17) Sets up a patient with ventilatory support.
- (18) Performs surfactant therapy.
- (19) Sterilizes respiratory care equipment.

4. RELATED CIVILIAN TRAINING.

- a. Course length. 24 months for entry level program (Associate Degree), and 48 months for an advanced level program (Bachelor Degree).
- b. Degree awarded. Associate of Science in Respiratory Care or Bachelors of Science in Respiratory Care.

c. Areas of clinical competency. The CoARC accredits entry into professional practice programs in respiratory care at the Associate, Baccalaureate, and Master's Degree level in the United States and its territories. Accreditation Standards for Entry into Respiratory Care Professional Practice, CoARC, revised June 2015. Endorsed by the AARC, ACCS, ATS, and ASA.

5. LICENSING/CREDENTIALING.

a. Army standard/requirement(s) for licensure and/or certification. None.

b. National and State requirements. 48 States require CRT entry level certification and two states require RRT advanced level certification for state licensure in order to practice as a respiratory therapist.

c. Additional requirements after completion of accredited course. None.

d. Degree requirements. Associate's degree required by CoARC effective January 2001 to be eligible to take the CRT examination.

e. Licensing/certification examination. The NBRC is a voluntary health certifying board which was created to evaluate the professional competence of respiratory therapists. The NBRC administers the TMC examination for the CRT credential and the Clinical Simulation Examination for those who are RRT eligible. The NBRC allows for several other credentials as well: ACCS, CPFT, NPS, RPFT, and SDS.

6. CONTINUING EDUCATION REQUIREMENTS.

a. Army standard/requirement(s). None.

b. Maintain civilian license/certification. Varying requirements based on State licensing board.

APPENDIX U – HEALTH CARE SPECIALIST (68W)

1. JOB DESCRIPTION. Provides emergency medical treatment, limited primary care, force health protection, evacuation in a variety of operational and clinical settings from point of injury or illness through the continuum of health care and provides emergency medical care/treatment at point of wounding on the battlefield or to battle and non-battle casualties. In the MTF, the 68W will assist in providing medical care under the supervision of a privileged, licensed, certified, and/or registered individual (i.e. physician, nurse practitioner, physician assistant, or RN). Additionally, the functions performed by these Soldiers will focus on the development of skills that will allow them to function independently in austere environments,

2. MILITARY COURSE. METC, JBSA Fort Sam Houston, TX; The Army School System Battalion, U.S. Army Reserve Command Region A, B, C, D, E, F, G.

a. Course length. Initial Entry Training Active Army (AA): 16 weeks. Reclassification Training Reserve Component (RC): To be determined.

b. Accreditation. National accreditation from the NREMT.

c. American Council on Education credit hours. Up to 37 depending on skill level.

d. School affiliation. None.

e. Degree awarded. None.

f. Eligibility for certification or licensing. Trainees are required to take and pass the NREMT certification examination administered by NREMT, and to maintain NREMT certification.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Enlisted Health Care Specialists (68W), are expected to perform all of their critical tasks while deployed, where they may not have direct medical supervision or connectivity to ask for advice or training. These personnel require the highest level of proficiency in all of their critical tasks to ensure readiness. The Medical Director (as identified in paragraph 6a (1) of this regulation) in conjunction with the Education and Training Department will ensure that Soldiers assigned to the MTF as permanent party or as Medical Skills Readiness (MSR) students receive the opportunity to meet all of the required critical task training consistent with the patient population and capabilities of the facility.

b. Licensed Independent Providers/ Privileged Healthcare Providers must **directly supervise** in-hospital procedures by those that have completed local MTF competency-based orientation and training.

(1) Per provider order, administers and records appropriate immunizations for adults. Allowed to administer immunizations to children less than 17 years of age only after completing the Y8 Immunization Course, or equivalent instruction as indicated in note below.

(2) Per provider order, administers and records appropriate allergy shots or skin testing after completion of the Y8 course, or equivalent instruction as indicated in note below. 68Ws who have not attended the 68WY8 course, or equivalent instruction, will not administer skin testing or allergy desensitization shots.

Note: Equivalent instruction to the Y8 course must have course learning objectives, specify content to be taught in the course, contain safety and appropriate restraint procedures, require a knowledge test of individual vaccines and vaccination protocols, require psychomotor observation using a competency checklist obtained from the Y8 course or from the immunization block on the Centers for Disease Control and Prevention website (www.cdc.gov), and describe the caliber of individuals instructing/supervising the course who must be licensed health care personnel and/or previous Y8 instructors.

(3) Per Licensed Independent Provider/ Privileged Healthcare Provider or licensed professional order, assists with the administration, recording, and evaluation of a patient's response to medication. Below are examples of the medication classes that the 68W must be familiar with (IAW tactical combat casualty care guidelines), in order to operate effectively on the battlefield. Training should focus on these types of medications, to ensure the Soldier's readiness to perform their battlefield mission.

(a) Topical ointments/lotions: Silver sulfadiazine, Hydrocortisone, Bacitracin, Neomycin, and Antifungal.

(b) Otic.

(c) Ophthalmic.

(d) Inhalation medications to include cardiac care protocols (when certified).

(e) Analgesics: Ketamine (limited to intranasal formulation), Fentanyl (limited to oral lozenge product), Meloxicam, and Morphine.

(f) Antibiotics: Cefotetan/Ertapenem, Levofloxacin, Cefazolin, and Ceftriaxone.

(g) Antiemetics: Ondansetron and Promethazine.

(h) Antifibrinolytics (Tranexamic Acid).

(i) Over-the-Counter (OTC).

(j) Antihistamines.

(k) All medication protocols associated with additional certifications or licensing for 68W (e.g. ACLS).

(4) All medication training will stress the importance of patient safety, focusing on the Five Rights of Medication Administration (right patient, right medication, right time and frequency of medication, right dose, and right route). Training must also include knowledge of therapeutic effect, adverse side effects, interactions, and contraindications. Additional training on medication administration may be considered based on patient population and the formulary of the particular MTF.

(5) Assists Licensed Independent Providers/ Privileged Healthcare Providers to perform invasive procedures per order and local MTF competency-based orientation and training, requiring annual skills validation (per paragraph 8e of this regulation).

(6) Assists in childbirth.

(7) Sets up and maintains a sterile field.

(8) Performs proper airway management for respiratory distress to include placement of supraglottic airways.

(9) Performs oral and nasogastric suctioning.

(10) Inserts and monitors nasogastric tube for content, output, and placement and performs gastric lavage as instructed.

(11) Performs suturing.

(12) Applies splints.

(13) Obtain a throat culture.

(14) Removes skin warts on extremities as ordered.

c. Procedures requiring indirect supervision:

(1) Patient screening (basic understanding of normal growth and development, age appropriate verbiage, and ability to discriminate between normal and abnormal values).

(a) Document patient care using Subjective, Objective, Assessment, Plan (SOAP) note format.

(b) Performs and documents physical screening and level of consciousness. Obtains and documents basic patient history to include medication history, prior surgery, social history, allergies, past medical problems, and pain history. Identifies/reports emergent/urgent conditions from history. Utilizes only approved abbreviations.

(c) Performs and records patient screening per protocol including pain, tobacco/alcohol use, depression, abuse, barriers to learning, and patient literacy. Identifies/reports medical/traumatic emergencies and normal/abnormal findings.

(d) Obtains, interprets, and documents temperature, heart rate, respiratory rate, pulse oximetry, and blood pressure. Identifies/reports normal/abnormal results.

(e) Assesses visual acuity with eye chart and gross hearing function.

(f) Monitors and records intake and output and appearance of output. Identifies/reports normal/abnormal results.

(g) Assesses gross motor function and strength.

(2) On-order interventions:

(a) Initiates, monitors, and discontinues intravenous/intraosseous infusion or saline lock.

(b) Controls bleeding by applying tourniquets, pressure bandages, pressure points, wound packing, and splints.

(c) Identifies, reports and treats hypovolemia.

(d) Utilizes proper technique in the initial management of fractures and initiates spinal precautions in suspected spinal injuries.

(e) Assists in the initial treatment of environmental injuries.

(f) Performs, interprets, records, and reports blood glucose levels. Identifies and reports hypoglycemia or hyperglycemia.

(g) Performs initial management of seizures.

(h) Recognizes indications for, provides and monitors by clinical assessment and pulse oximetry, the use of supplemental oxygen via nasal cannula, oxygen mask, pocket mask, bag-valve mask device supraglottic and endotracheal airways. Identifies/reports hypoxia and hypercarbia.

(i) Examines eye using fluorescein strip.

(j) Performs ear and eye irrigation (using age-appropriate techniques).

(k) Obtains laboratory specimens to include blood (venipuncture, capillary), urine, and cultures (wound, stool, urine, throat, sputum, and blood). Utilizes two patient identifiers in collection process.

(l) Performs appropriate wound care using sterile technique to include dressing changes, wound cultures, and wound irrigation.

(m) Performs pre-hospital and intra-hospital patient transport in accordance with medical director order and EMT level of certification. Provides appropriate information during patient or specimen hand-offs.

(n) Gathers/processes sexually transmitted disease specimen from genital smears.

(o) Performs 12-lead electrocardiogram. Places patient on a cardiac monitor. Identifies and reports normal or abnormal cardiac rhythms.

(p) Identifies and reports emergency cardiac dysrhythmias. Provides emergency treatment according to level of certification.

(q) Assists with the administration of medication through endotracheal tube.

(r) Assists with the administration of local anesthesia.

(s) Assists in performing digital block procedures.

(t) Perform toenail removal.

(u) Assists in performing a needle chest decompression.

(v) Assist with the insertion of a chest tube thoracotomy.

(w) Assist with the administration of blood products.

(3) Procedures focused to promote patient safety:

(a) Identifies potentially dangerous care situations ("near miss events") (see glossary) and reports according to local policy.

(b) Questions unsafe patient care activities.

(c) Incorporates safety into all aspects of patient care.

(d) Practices in accordance with guidelines and local policies avoiding unsafe short-cuts.

4. RELATED CIVILIAN EQUIVALENT TRAINING.

- a. Course length. EMT 110 hours minimum
- b. Degree awarded. None.
- c. Areas of clinical competency. Reference the cognitive, affective, and psychomotor skills as identified under the National Highway Traffic Safety Administration (NHTSA) - The National Standard Curriculum for the National Registry of Emergency Medical Technicians (NREMT); the Prehospital Trauma Life Support (PHTLS) protocols; the Tactical Combat Casualty Care – Medical Provider (TCCC-MP) guidelines; and the Advanced Cardiac Life-Saving (ACLS) Protocols.

5. LICENSING/CREDENTIALING.

a. U.S. Army EMS is recognized by the National Registry of Emergency Medical Technicians (NREMT) as the State EMS office for the Army. It is the state oversight and accrediting body for all 68Ws under the NREMT certification model. The Medical Director for Army EMS along with NREMT has determined that the scope of practice for the 68W is well defined within the TC 8-800 sustainment model and the most current STP 8-68W13-SM-TG. This is the scope of practice for which 68Ws will train and be utilized. Failure to meet standard of care for NREMT is reportable to the National Practitioner Data Bank.

Army standard/requirement(s) for licensure and/or certification. National Registry Emergency Medical Technician certification; health care provider CPR; Tactical Combat Casualty Care-Medical Provider (TCCC-MP); Advanced Cardiac Life-Saving (ACLS).

- b. National and State requirements. None.
- c. Degree requirements. None.
- d. Licensing/certification examination. NREMT

6. CONTINUING EDUCATION REQUIREMENTS.

a. Army standard/requirement(s) for licensure and/or certification. Must maintain NREMT recertification requirements.

b. Maintain civilian license/certification. The following identifies the Combat Medical Specialist EMT recertification requirements (detailed instructions in TC 8-800):

c. TC 8-800 Medical Education Demonstration of Individual Competence Tables 1 through 8 every year, Basic Life Saving (BLS) certification with recertification packet to U.S. Army Emergency Medical Services (EMS).

APPENDIX V – FLIGHT PARAMEDIC SPECIALTY (68WF2)

1. JOB DESCRIPTION. Provides emergency medical treatment and air evacuation in a variety of operational environments from point of injury or illness, Role I/II care and critical care transport to Role II/III/IV; administers emergency medical treatment to battle and non-battle casualties; develops and trains soldiers through MOS enhancement. Provides patient assessment, emergency care, and post-surgical care during transport to a higher echelon of care. As a foundation, the Critical Care Flight Paramedic (CCFP) will maintain knowledge and skill proficiency at a National Registry Paramedic (NRP) level.

2. MILITARY COURSE. Critical Care Flight Paramedic Program, Center for Pre-hospital Medicine, AMEDDC&S HRCOE, JBSA Fort Sam Houston, TX.

a. Flight Paramedic.

- (1) Course length. 26 weeks 2 days.
- (2) Accreditation. National accreditation from the Committee on Accreditation of Educational Programs for the EMS Professions (CoAEMSP) as a paramedic program.
- (3) American Council on Education credit hours. N/A.
- (4) School affiliation. Commission on Accreditation of Allied Health Education Program-approved (CAAHEP) course.
- (5) Degree awarded. None. National Registered Paramedic certification.

b. Critical Care Paramedic.

- (1) Course length. 8 weeks 2 days.
- (2) Accreditation. School approved by CCFPP Medical Director.
- (3) American Council on Education credit hours. N/A.
- (4) School affiliation. None.

c. Army Medical Department (AMEDD) Aviation Crewmember.

- (1) Course length. 4 weeks.
- (2) Accreditation. None.
- (3) American Council on Education credit hours. N/A.
- (4) School affiliation. None.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Medications: Per provider order, local medical director protocols, and the Standard Medical Operating Guidelines (SMOG), administers, records, and evaluates response to the emergency medication IAW current SMOG.

b. Neurological patient. Perform neurological assessment and treatment of point of injury and critical care patient. Manage head and/or spinal cord injury IAW current SMOG and local protocols.

- (1) Manage neurologic patient's pharmacological agents.
- (2) Interpret neurologic patient's lab values.
- (3) Perform cranial nerve assessment.
- (4) Operate/troubleshoot intracranial pressure monitoring systems.

c. Cardiac patient. Perform cardiac assessment and treatment of point of injury and critical care patient. Manage injury/illness IAW current SMOG, Advanced Cardiac Life Support (ACLS) guidelines, and local protocols.

- (1) Manage cardiac patient's status using available diagnostic equipment.
- (2) Manage cardiac patient's status using pharmacological agents.
- (3) Manage cardiac patient's status using laboratory values.
- (4) Operate arterial line.
- (5) Use invasive monitoring during transport of cardiac patient.
- (6) Manage cardiac patient's status using hemodynamic parameters.
- (7) Operate intravenous pump.
- (8) Manage a cardiac patient with an intra-aortic balloon pump.

d. Respiratory patient. Perform respiratory assessment and treatment of point of injury and critical care patient. Manage respiratory patient IAW current SMOG and local protocols.

- (1) Perform an assessment of the post-surgical respiratory patient.
- (2) Manage respiratory patient's status using available diagnostic equipment.
- (3) Manage respiratory patient's status using pharmacological agents.
- (4) Manage respiratory patient on a mechanical ventilator.
- (5) Perform Rapid Sequence Endotracheal Intubation on a respiratory patient.

e. Toxic exposure patient. Perform assessment and treatment of point of injury and critical care patient involved in a toxic exposure. Manage patient IAW current SMOG and local protocols.

- (1) Manage a contaminated patient's status using diagnostic equipment.
- (2) Manage toxic exposure patient with pharmacological agents.

f. Environmental patient. Perform assessment and treatment of point of injury and critical care environmental patient. Manage patient IAW current SMOG and local protocols.

- (1) Provide care for a patient of envenomation.
- (2) Manage an environmental patient's status using diagnostic equipment.
- (3) Manage an environmental patient's status using laboratory values.
- (4) Manage an environmental patient's pharmacological agents.

g. Obstetrical patient. Perform assessment and treatment of point of injury and critical care obstetrical patient. Manage patient IAW current SMOG and local protocols.

- (1) Manage an obstetrical patient's status using diagnostic equipment.
- (2) Manage an obstetrical patient's status using laboratory values.
- (3) Manage pharmacological agents for an obstetrical patient.
- (4) Manage a precipitous delivery for an obstetrical patient.
- (5) Manage acute complications of precipitous delivery for an obstetrical patient.

h. Neonatal/pediatric patient. Perform assessment and treatment of point of injury and critical care pediatric/neonatal patient. Manage patient IAW current SMOG and local protocols.

- (1) Manage a pediatric/neonatal patient's status using diagnostic equipment.
- (2) Manage pediatric/neonatal patient's pharmacological agents.
- (3) Manage a pediatric/neonatal patient's status using laboratory values.

i. Burn patient. Perform assessment and treatment of point of injury and critical care burn patient. Manage patient IAW current SMOG and local protocols.

- (1) Manage a burn patient's status using diagnostic equipment.
- (2) Manage burn patient's pharmacological agents.

- (3) Manage a burn patient's status using laboratory values.

j. General medical patient. Perform assessment and treatment of point of injury/illness and critical care general medical patient. Manage patient IAW current SMOG and local protocols.

- (1) Manage a medical patient's status using diagnostic equipment.
- (2) Interpret radiographic results.
- (3) Manage a medical patient's status using laboratory values.
- (4) Use invasive monitoring for the purpose of clinical management.
- (5) Manage a sepsis patient's pharmacological agents.
- (6) Manage endocrine system complications.
- (7) Manage renal and urology systems complications.
- (8) Manage gastrointestinal system complications.
- (9) Manage gastrointestinal patient's pharmacological agents.
- (10) Develop a treatment plan to maintain a critical care medical patient.

k. General trauma patient. Perform assessment and treatment of point of injury/illness and critical care general trauma patient. Manage patient IAW current SMOG and local protocols.

- (1) Perform pre-hospital ultrasound assessment on a trauma patient (FAST).
- (2) Manage a patient receiving blood transfusion.

l. Military Working Dog. Perform assessment and treatment of point of injury/illness and critical care of military working dog. Manage patient IAW current SMOG and local protocols.

- (1) Determine a military working dog's vital signs.
- (2) Perform a physical exam on a MWD.
- (3) Intubate a military working dog.
- (4) Manage a military working dog with heat injury.
- (5) Manage a military working dog with dehydration.
- (6) Manage a military working dog with an allergic reaction.

- (7) Manage a military working dog with hypovolemic shock.
- (8) Manage a military working dog with gastric-dilatation volvulus.
- (9) Perform basic cardiac life support on a military working dog.
- (10) Manage a military working dog with an open chest wound.
- (11) Manage a military working dog with an open abdominal wound.
- (12) Manage a military working dog with an external hemorrhage.
- (13) Perform a needle thoracentesis on a military working dog.
- (14) Perform a thoracostomy tube placement in a military working dog.
- (15) Administer analgesics to a military working dog.

4. RELATED CIVILIAN EQUIVALENT TRAINING.

- a. Course length. 1,000 hours of initial training for EMT-P.
- b. Degree awarded. None.

5. LICENSING/CREDENTIALING.

a. Army standard/requirement(s) for licensure and/or certification. Military Occupational Specialty (MOS)-qualified (68W), Class III Flight Physical, National Registry Paramedic (NRP) certification, Advanced Cardiac Life Support certification and health care provider CPR certification. Failure to meet standard of care for NREMT is reportable to the National Practitioner Data Bank.

b. National and State requirements. NRP certification, ACLS certification, and health care provider CPR certification.

c. Degree requirements. None.

d. Licensing/certification examination. National Registry Paramedic (NRP) certification to National Registry of Emergency Medical Technicians (NREMT).

6. CONTINUING EDUCATION REQUIREMENTS.

a. Army standard/requirements(s) for licensure and/or certification. Must maintain NRP recertification requirements to include ACLS and CPR.

b. Maintain mandatory certification. The following identifies the 68WF2's recertification requirements.

(1) Refresher course. The Paramedic (ASI F2) Recertification course (300-NREMT-P/NRP Recert) ATRRS listed, 72 hours minimum every 2 years; mandatory recertification (ACLS, CPR) and Pediatric Advanced Life Support (PALS) achieved.

(2) Skills validation. IAW NREMT policies and Army EMS standards, 72 hours minimum every 2 years; conducted at unit level by 68WF2 Standardization Instructor (SI), 68WF2 Flight Instructor (FI), or personnel designated by Aircrew Training Program (ATP) commander.

(3) Health provider CPR. 8 hours every 2 years. Required certification to maintain NRP certification.

7. MOS REQUIREMENT – NREMT CERTIFICATION. Soldiers (AA/RC) possessing both the 68W MOS and F2 ASI are required to obtain and maintain certification as NRPs. Certification and requirements will be IAW NREMT policies and Army EMS standards. Soldiers who fail to recertify will lose MOS/ASI qualifications after 90 calendar days (for AA) and 180 calendar days (for RC and NG). The Soldier will require 100% supervision during the non-certified period, or removal from patient care until certification is obtained. A Soldier's failure to obtain NRP certification immediately following the respective 90- or 180-day period will result in the initiation of an appropriate personnel action (i.e, PCS, mandatory reclassification, separation) according to governing regulations.

APPENDIX W – BEHAVIORAL HEALTH SPECIALIST (68X)

1. JOB DESCRIPTION. Facilitate BH activities in Garrison and Operational environments. Assist in conducting psycho-educational classes and groups, administering medication. Assisting in the treatment of clients with BH diagnoses or symptoms that include, but are not limited to substance use disorders, trauma and stressor related disorders and other BH disorders. Only 68Xs with the M8 Additional Skill Identifier (Drug and Alcohol Counselor) can participate in treatment of clients with substance-related and addictive disorders. Under the direct supervision of a Licensed Independent Provider/ Privileged Healthcare Provider, conduct initial interviews, triage, risk assessments, client assessments, mental status examinations, psychological testing, develop and implement treatment plans, group facilitation, case management, collect and record psychosocial and physiological data, and other instruments that assess the client's potential or actual response to treatment and/or rehabilitation. Combat and Operational Stress Control Operations.

2. MILITARY COURSE. AMEDDC&S HRCoE, JBSA Fort Sam Houston, TX.

- a. Course length. 17 weeks and 4 days.
- b. Accreditation. None.
- c. American Council on Education credit hours. 6 credit hours. Community College of the Air Force (CCAF): 19 credit hours
- d. School affiliation. None.
- e. Degree awarded. None.
- f. Eligibility for certification or licensing. Field Traumatologist through the Academy of Traumatology Commission on Certification and Accreditation (COCA); Certified Psychiatric Rehabilitation Practitioner (CPRP); Level I or Level II Nationally Registered Mental Health Technicians (depending on qualifications) with the Professional Psychologist Certification Board; certification with the American Psychiatric Technicians Association; Traumatic Event Management basic and advanced certification; biofeedback technician; Nursing Board Licensed Psychiatric Technician (Arkansas, Colorado, California or Kansas); M8 Drug and Alcohol Counselor ASI.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

- a. Procedures requiring direct supervision.

- (1) Assess a Client's Psychosocial Functioning.
 - (2) Facilitate Referral Services for Behavioral Health Clients.
 - (3) Assess Client Psychopathology.
 - (4) Assess a Client's Mental Status.
 - (5) Assess Behavioral Health Client for Substance Use.
 - (6) Conduct a Counseling Session.
 - (7) Administer Psychological Testing.
 - (8) Conduct a Collateral Interview.
 - (9) Employ Behavioral Health De-Escalation Techniques.
 - (10) Co-facilitate a Group Therapy Session.
 - (11) Conduct Behavioral Health Case Management.
 - (12) Perform a Behavioral Health Risk Assessment.
 - (13) Conduct an Intake Interview.
 - (14) Conduct Behavioral Health Triage.
 - (15) Conduct Behavioral Health Treatment Team Meetings.
 - (16) Perform Behavioral Health Needs Assessment.
 - (17) Implement Unit Reconstitution Support.
 - (18) Provide Behavioral Health Rehabilitation Team Meetings.
 - (19) Coordinate Behavioral Health Support in Response to a Potentially Traumatic Event.
- b. Procedures requiring indirect supervision.
- (1) Perform Clinical Procedures in Behavioral Health Settings.

- (2) Develop Combat and Operational Stress Control Support Plan.
- (3) Maintain Compliance with Behavioral Health Ethical Guidelines within Behavioral Health Operations.
- (4) Conduct Behavioral Health Consultation.
- (5) Perform Tele-Behavioral Health (TBH) Services.
- (6) Conduct Psycho-educational Brief.
- (7) Conduct Behavioral Health Outreach.

4. RELATED CIVILIAN TRAINING. No equivalent as civilian psychiatry technicians have less training and responsibilities than Army 68X Specialists.

- a. Course length. 12-14 months.
- b. Degree awarded. None.
- c. Areas of clinical competency. See website at <https://www.cool.army.mil/>

5. LICENSING/CREDENTIALING.

- a. Army standard/requirement(s) for licensure and/or certification. None.
- b. National and State requirements. None.
- c. Degree requirements. None.

6. CONTINUING EDUCATION REQUIREMENTS.

- a. Army standard/requirement(s). None.
- b. Maintain civilian license/certification. None.

APPENDIX X – DRUG AND ALCOHOL COUNSELING SPECIALIST (68XM8)

1. JOB DESCRIPTION. Provide drug and alcohol preventive education, substance use disorder assessments, counseling and supportive care both in garrison and deployed.

2. MILITARY COURSE. USADART Individual Counseling Course; USADART Group Counseling Course.

a. Course length. 4 weeks (2 weeks for each course).

b. Accreditation. None.

c. American Council on Education credit hours. None

d. School affiliation. None.

e. Degree awarded. None.

f. Eligibility for certification or licensing. The Individual and Group course together contribute required Drug and Alcohol related curriculum (135 hours) towards International Certification & Reciprocity Consortium (IC&RC) certification. The courses do not meet the practicum requirements. Each state has specific prerequisites in addition to IC&RC to complete credentialing requirements.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision. None.

b. Procedures requiring indirect supervision. None.

4. RELATED CIVILIAN TRAINING.

a. Course length. 1.5 years didactic.

b. Degree awarded. None.

c. Areas of clinical competency. Standards for training are set by IC&RC and related member boards.

5. LICENSING/CREDENTIALING.

a. Army standard/requirement(s) for licensure and/or certification. None.

b. National and State requirements. None.

c. Additional requirements after completion of accredited course. Completion of the Individual and Group courses contributes 135 of 270 CEUs required for the IC&RC exam. An additional minimum 300 hours of clinical practicum are also required, as set by each IC&RC member board.

d. Degree requirements. None.

e. Licensing/certification examination. Voluntary IC&RC certification examination is provided at various locations and at multiple scheduled times. Information may be obtained about the IC&RC certification at phone (717) 540-4457, Fax (717) 773-4483, email info@internationalcredentialing.org or IC&RC website <http://internationalcredentialing.org/>.

6. CONTINUING EDUCATION REQUIREMENTS.

a. Army standard/requirement(s). None.

b. Maintain civilian license/certification. None.

APPENDIX Y - EYE SPECIALTY (68Y)

1. JOB DESCRIPTION. To conduct routine diagnostic tests and assist in care and treatment of optometry/ophthalmology patients under the supervision of an optometrist/ophthalmologist.

2. MILITARY COURSE. Phase 1: METC, JBSA Fort Sam Houston, TX;
Phase 2: Eight sites.

a. Course length. 13 weeks (Phase 1: 6 weeks; Phase 2: 7 weeks).

b. Accreditation. Joint Commission on Allied Health Personnel in Ophthalmology (JCAHPO). Renewed on an annual basis by submission of annual report detailing any changes to the course curriculum.

c. American Council on Education credit hours. 10 credit hours.

d. School affiliation. Community College of the Air Force credit hours. 10.

e. Degree awarded. None.

f. Eligibility for certification or licensing. Certified ophthalmic assistant (COA) – eligible after 12 months or 500 hours in clinic under ophthalmology supervision. If certification is not awarded within 36 months of completing the 323-68Y10 eye specialty course, the 68YP3 must complete 18 continuing education credits approved by JCAHPO prior to applying for COA certification.

3. CLINICAL BASELINE COMPETENCY TASK LIST.

a. Procedures requiring direct supervision by ophthalmologist or optometrist.

- (1) Assists ophthalmologist during surgery.
- (2) Administers treatment for ocular injuries.
- (3) Administers treatment for ocular burns.
- (4) Administers treatment for ocular diseases.

b. Procedures requiring indirect supervision.

- (1) Prepares for ocular surgical procedures.

- (2) Conducts patient screenings.
- (3) Initiates treatment for a chemical burn of the eye.
- (4) Removes non–embedded foreign bodies from the conjunctiva or cornea.
- (5) Conducts visual field tests.
- (6) Performs ultrasound ocular scans.
- (7) Performs ocular photography.
- (8) Triage eye emergencies.
- (9) Dispenses contact lenses.
- (10) Orders, dispenses, adjusts, and repairs prescription eyewear.

4. RELATED CIVILIAN TRAINING.

- a. Course length. The most closely related course is 1 year in length and graduates are eligible for certification.
- b. Degree awarded. Associate of Applied Science in Vision Technology.
- c. Areas of clinical competency.
 - (1) Medical histories.
 - (2) Diagnostic testing.
 - (3) Refraction.
 - (4) Anatomical/functional ocular measurements and tests.
 - (5) Administration of topical ophthalmic and oral medications.
 - (6) Assist in minor ophthalmic surgery.

5. LICENSING/CREDENTIALING.

- a. Army standard/requirements.

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b. National and State requirements. There are currently no certification requirements to work as an ophthalmic technician.

c. Additional requirements after completion of accredited course. Individuals with a high school diploma or equivalent are eligible to take the JCAHPO COA examination if all of the following criteria are fulfilled:

(1) Completion of a Committee on Accreditation for Ophthalmic Medical Personnel approved independent study course (e.g., American Academy of Ophthalmology Independent Study Course or the Canadian Home Study Course).

(2) Completion of 1-year full-time work in a clinic as an ophthalmic assistant under the supervision of an ophthalmologist. (Completion of an accredited resident ophthalmic assistant course is not required by JCAHPO for COA certification.)

NOTE: Requirements for the progressive levels of certification are listed by JCAHPO certification booklet (www.jcahpo.org, 2025 Woodlawn Drive, St. Paul, MN 55125- 2995).

d. Degree requirements. None.

d. Licensing/certification examination. The certification examination is composed of 200 multiple-choice questions administered during three hours. JCAHPO awards the COA credential to the applicants that achieve a passing score.

6. CONTINUING EDUCATION REQUIREMENTS.

a. Army standard/requirement. None.

b. Maintain civilian license/certification. COAs must obtain 18 continuing education credits every 3 years to recertify as a COA.

7. MOS REQUIREMENT - NONE.

Glossary

Section I Abbreviations

AA.....	Active Army
AAB	Association of Bio analysts
ABO.....	American Board of Opticianry
ACE	American Council on Education
ACLS.....	Advanced Cardiac Life Support
ADTMC.....	Algorithm-Directed Troop Medical Care
AMEDDC&S.....	U.S. Army Medical Department Center and School
AMT.....	American Medical Technology
AOC	Area of Occupational Concentration
ARDMS	American Registry for Diagnostic Medical Sonography
ARRT.....	American Registry of Radiologic Technologist
ASCP.....	American Society of Clinical Pathologists
ASHP	American Society of Health System Pharmacists
ASI.....	Additional Skill Identifier
ASOP	American Society of Orthopedic Professionals
AST.....	Association of Surgical Technologists
ATP	Aircrew Training Program
ATRRS.....	Army Training Requirements and Resources System
BCCTCP	Board for Critical Care Transport Paramedic Certification
BLS.....	Basic Life Support
BOC	Board of Certification
BOR.....	Board of Registry
CAAHEP.....	Commission on Accreditation of Allied Health Education Programs
CAF	Competency Assessment Folder
CAOHC.....	Council for Accreditation in Occupational Hearing Conservation
CAPTE	Commission on Accreditation in Physical Therapy Education
CCAF	Community College of the Air Force
CCFP	Critical Care Flight Paramedic
CCI.....	Cardiovascular Credentialing International
CCP	Certification Continuation Program
CCP-C	Critical Care Paramedic Certification
CDA.....	Certified Dental Assistant
CDM-CFFP	Certified Dietary Manager, Certified Food Protection Professional
CE	Continuing Education
CEHRS	Certified Electronic Health Record Specialist
CEU	Continuing Education Unit

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CLT.....	Clinical Laboratory Technician
CME	Continuing Medical Education
CMF.....	Career Management Field
CMP	Credential Maintenance Program
COA.....	Certified Ophthalmic Assistant
CoAEMSP	Committee on Accreditation of Educational Programs for the EMS Professions
CoARC.....	Committee of the Accreditation of Respiratory Care
CODA	Commission on Dental Accreditation
COE	Council on Occupational Education
CPhT	Certified Pharmacy Technician
CPR.....	Cardiopulmonary Resuscitation
CPRP	Certified Psychiatric Rehabilitation Practitioner
CQMP.....	Clinical Quality Management Program
CRT.....	Certified Respiratory Therapy
CST.....	Certified Surgical Technologist
DANB.....	Dental Assistants National Board
DOD.....	Department of Defense
DOT.....	Department of Transportation
DOTAA	Director of Training and Academic Affairs
DTR	Dietetic Technician, Registered
EMS	Emergency Medical Services
EMT.....	Emergency Medical Technician
FTE	Full Time Equivalent
FP-C	Flight Paramedic Certification
HRCoE	Health Readiness Center of Excellence
IAC	International Academy of Cytology
IAW	In Accordance With
IC&RC	International Certification & Reciprocity Consortium
JBSA	Joint Base San Antonio
JCAHPO.....	Joint Commission on Allied Health Personnel in Ophthalmology
JRC-CVT	Joint Review Committee on Education in Cardiovascular Technology
LIS	Laboratory Information Systems
LPN.....	Licensed Practical Nurse
LVN.....	Licensed Vocational Nurse
MEDCOM.....	U.S. Army Medical Command
METC	Military Education and Training Campus
MHS.....	Military Health System
MOS.....	Military Occupational Specialty
MQA	Medical Quality Assurance
MSR.....	Medical Skills Readiness

MTF.....	Military Treatment Facility
NAACLS.....	National Accrediting Agency for Clinical Laboratory Sciences
NBCOT.....	National Board for Certification of Orthopedic Technologists
NBRC	National Board for Respiratory Care
NBSTA	National Board of Surgical Technology and Surgical Assisting
NCA.....	National Credentialing Agency
NCLE	National Contact Lens Examination
NCLEX-PN.....	National Council Licensure Examination - Practical Nurse
NHA	National Healthcareer Association
NHTSA	National Highway Traffic Safety Administration
NMTCB.....	Nuclear Medicine Technology Certification Board
NREMT.....	National Registry of Emergency Medical Technicians
NRP	National Registry Paramedic
OTA	Occupational Therapy Assistant
OTR	Registered Occupational Therapist
PCS.....	Permanent Change of Station
PDU	Professional Development Units
PHTLS.....	Pre-Hospital Trauma Life Support
PMOS.....	Primary Military Occupational Specialty
PTA.....	Physical Therapy Assistant
PTCE	Pharmacy Technician Certification Exam
RC.....	Reserve Component
RCIS.....	Registered Cardiovascular Invasive Specialist
RDCS	Registered Diagnostic Cardiac Sonographer
RDA.....	Registered Dental Assistant
ROT	Registered Orthopedic Technician
RRT.....	Registry Respiratory Therapist
ACMS-VT.....	Annual Combat Medic Skills Validation Test
SCT	Cytotechnology Specialist
SI	Skill Identifier
SICP	Society of Invasive Cardiovascular Professionals
SMOG	Standard Medical Operating Guidelines
SO-ATP	Special Operations-Advanced Tactical Paramedic
SOCOM	Special Operations Command
SOF	Special Operations Forces
SOFMSSP.....	Special Operations Forces Medical Skills Sustainment Program
STP	Soldier Training Publication
TBH	Tele-Behavioral Health
TC.....	Training Circular
TCCC	Tactical Combat Casualty Care

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TCCC-MP Tactical Combat Casualty Care – Medical Provider
TJC.....Joint Commission on Accreditation of Healthcare Organizations
TMEP Tactical Emergency Medical Protocols
TOE.....Table of Organization and Equipment
TP-C Tactical Paramedic Certification
USASOC.....U.S. Army Special Operations Command

Section II Terms

Accountability

Being responsible for actions or inactions of self and others in the context of delegation.

Algorithm

A systematic process consisting of an ordered sequence of steps, each step depending on the outcome of the previous step.

U.S. Army Medical Department Center and School, U.S. Army Health Readiness Center of Excellence.

The academic accreditation organization that approves enlisted technical training and standards.

Certification

Official recognition of an individual by a national agency or association that is intended to assure the public that the health care professional has successfully completed an approved educational program and evaluation. This includes a formal examination designed to assess the knowledge, experience, and skills requisite to the provision of high-quality patient care in that specialty.

Competence

The ability to perform the duties, functions, and requirements of a particular discipline, job, or duty position as measured by meeting the following conditions:

1. Authorized to practice a specified scope of care under a written plan of supervision at any time within the past 2 years, or completed formal graduate professional education in a specified clinical specialty at any time within the past 2 years, or privileged to practice/authorized to provide a specified scope of care at any time within the past 2 years.
2. Actively pursued the practice of his/her discipline, job, or duty position within the past 2 years by having encountered a sufficient number of clinical cases or variety of experiences to represent a broad spectrum of the privilege requested or scope of care authorized.
3. Satisfactorily practiced the discipline as determined by the results of professional staff monitoring and evaluation relative to the quality and appropriateness of patient care.

Core competence

Possession of the knowledge, skills, and abilities necessary to fulfill the job responsibilities. Clinical competencies for each MOS/ASI are listed in appendixes A through V of this regulation.

Delegation

To entrust to another competent individual the authority to perform a selected task(s) in a selected situation(s).

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Direct patient care activities

Those activities which assist the patient in various health care settings. This includes assisting with activities of daily living; therapeutic and diagnostic interventions; and collecting, reporting, and documenting data related to these activities.

Indirect patient care activities

Those activities necessary to support the patient and the environment of care such as preparing the patient for a procedure.

License

A grant of permission by an official agency of a State; the District of Columbia; or a Commonwealth, territory, or possession of the United States to provide health care within the scope of practice of a specified discipline. Licenses must be--Current. Active, not revoked, suspended, or lapsed in registration.

1. Active. Characterized by present activity, participation, practice, or use.
2. Valid. The issuing authority accepts, investigates, and acts upon quality assurance information, such as practitioner professional performance, conduct, and ethics of practice, regardless of the practitioner's military status or residency.
3. Unrestricted. Not subject to limitations on the scope of practice ordinarily granted all other applicants for similar specialty in the granting jurisdiction.

Near Miss.

An event or situation that could have resulted in harm to a patient but did not, either by chance or through timely intervention. The event was identified and resolved before reaching the patient. Such events have also been referred to as "close call incidents."

Protocol

A written procedure providing basic guidelines for the management (diagnosis and treatment) of specific types of medical or dental patient care in specified circumstances.

Skills Validation

Recognition by the AMEDDC&S HRCoE/MTF of the competence of an individual who has met predetermined qualifications and the authorization of the individual to perform certain procedures for which they have been trained or are otherwise qualified.

Supervision

The process of reviewing, monitoring, observing, and accepting responsibility for assigned personnel.

1. Direct. The supervisor is involved in the decision-making process. This may be further subdivided as follows:

2. Verbal. The supervisor is contacted by telephone or informal consultation before implementing or changing a regimen of care.
3. Physically present. The supervisor is present physically through all or a portion of care.
4. Indirect. The supervisor performs retrospective review of selected patient medical records. Criteria used for review are related to quality of care, quality of documentation, and the authorized scope of practice. Indirect supervision may also include countersignature of authentication of medical entries, reports, or orders prescribed by another.

Supervision (Enhanced)

Supervision afforded a provider with regular privileges for whom the need to assess competence and performance has been identified. This may be appropriate--

1. Following a PCS move or a provider's return to patient care responsibility from an administrative/nonclinical assignment.
2. During a period of temporary duty.
3. When privileges for a new procedure are granted.

Sustainment

The provision of training to support the maintenance of core competencies (identified by the MOS and/or ASI Branch, AMEDDC&S HRCOE) and unit-specific skills.

The proponent of this publication is the Office of the Assistant Chief of Staff for Health Policy and Services. Users are invited to send comments and suggested improvements on DA Form 2028 (Recommended Changes to Publications and Blank Forms) to Commander, U.S. Army Medical Command, ATTN: MCHO-CL, 2050 Worth Road, JBSA Fort Sam Houston, TX 78234-6010.

FOR THE COMMANDER:



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